PEDIATRIC MEDICINE · COMPLETE QUESTION BANK

Pediatric Medicine
All Lectures Question Bank

طب الأطفال — مجمّع أسئلة المادة كلها

5	237	32	60
Lectures	Past exam questions	NCLEX-style	Flashcards

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المصادر:

كويز 2 – 2024 (أطفال نظري) · كويز 2 – 2022 (أطفال نظري) · ميدترم 2021 (أطفال نظري) · ميدترم 2020 (أطفال نظري) · أسئلة سابقة · كويز 2 أطفال نظري · كويز 2 شعبة 3 (أطفال نظري) · كويز 2 شعبة 1 (أطفال نظري) · ميدترم 2024 (ترم أول 2023-2024) (أطفال نظري) · ميدترم 2022 (أطفال نظري)

ترقيم الأسئلة: رقم المحاضرة ثم رقم السؤال (مثلاً 4.12). أسئلة NCLEX عليها حرف N. الإجابات كلها في آخر الملف.

اللَّهُمَّ لا سَهْلَ إِلَّا ما جَعَلْتَهُ سَهْلًا، وأنتَ تَجْعَلُ الحَزْنَ إذا شِئْتَ سَهْلًا — صلِّ على النبي ﷺ

LECTURE 1

Ethics (Ethical Approach & Medicolegal Dilemmas)

الأخلاقيات والمعضلات الطبية القانونية

Part A · Past Exam Questions

أسئلة الدفعات السابقة

1.1 What is the most important ethical principle in pediatric care? أسئلة سابقة

a. Respecting the child's wishes
b. Acting in the best interest of the child
c. Treating everyone fairly
d. Always telling the truth

1.2 When can a child consent to medical treatment without parental permission? أسئلة سابقة

a. Never
b. If they understand the risks
c. If they are legally emancipated
d. Only when parents are present

1.3 What should a nurse do if they suspect child abuse? أسئلة سابقة

a. Ignore it until confirmed
b. Report it to the appropriate authorities immediately
c. Ask the child for confirmation
d. Wait for someone else to report it

1.4 When can a doctor make an urgent medical decision without parental consent? أسئلة سابقة

a. If the child does not want parents to know
b. If the child's condition is life-threatening
c. If the doctor is too busy to wait
d. If the child's friends request it

1.5 What is the main goal of the 'best interest principle' in pediatric decisions? أسئلة سابقة

a. Always following the doctor's opinion
b. Making the decision that benefits the child the most
c. Ensuring parents always decide
d. Letting the child decide without interference

1.6 In which situation can a child receive confidential medical treatment? أسئلة سابقة

a. Mild headache
b. Sexually transmitted infections (STIs)
c. Broken arm
d. Fever

1.7 Which legal document determines who can make medical decisions for a child if parents are unavailable? أسئلة سابقة

a. Medical will
b. Power of attorney
c. Parental consent form
d. Do-not-resuscitate (DNR) order

1.8 When treating a child with divorced parents, what is the key legal consideration? أسئلة سابقة

a. Only the custodial parent can consent
b. Both parents must always agree
c. Any parent with legal custody can consent
d. A court order is required

1.9 If parents disagree about their child's medical treatment, what usually happens? أسئلة سابقة

a. The doctor decides alone
b. The child is left untreated until parents agree
c. The court intervenes in significant cases
d. The child decides independently

1.10 Why don't children have full rights to make their own medical decisions? أسئلة سابقة

a. Children do not understand anything
b. Children have no legal rights
c. They may not have the full ability to make complex decisions
d. Only parents have the right to decide

1.11 A 14-year-old asks the nurse not to tell the parents about a sexual-health discussion. What is the most ethical action? أسئلة سابقة

a. Inform the parents immediately, as they are legally responsible
b. Respect the adolescent's privacy unless there is a safety concern
c. Refuse to discuss the topic without a parent present
d. Document it without informing the adolescent

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

1.N1 Which ethical principle means equal distribution of nursing care according to priorities and need? NCLEX

a. Autonomy
b. Justice
c. Fidelity
d. Veracity

1.N2 A family offers the nurse an expensive gift at discharge. The appropriate action is to: NCLEX

a. Accept it privately
b. Politely refuse the gift
c. Accept and share with colleagues
d. Ask the supervisor to keep it

1.N3 Select ALL the fundamental rights of the child mentioned in the lecture. SELECT ALL

a. Right to live
b. Right to privacy
c. Protection from danger
d. Right to treatment
e. Right to vote

1.N4 When approaching a frightened preschooler, the nurse should first: NCLEX

a. Start the examination quickly
b. Get down to the child's eye level and build trust with the parent present
c. Separate the child from the parent
d. Use medical terms

1.N5 Parents refuse a blood transfusion for their 6-year-old with life-threatening anemia. The priority action is: NCLEX

a. Respect the refusal and discharge
b. Seek a court order/intervention while documenting the refusal
c. Give the transfusion secretly
d. Transfer the child to another hospital

1.N6 Under Egyptian law, female genital mutilation requested by a family should be: NCLEX

a. Performed in a sterile setting
b. Refused, with counselling and reporting if harm was done
c. Referred to a private clinic
d. Done only with father's consent

1.N7 Select ALL situations where the child can be treated without parental consent. SELECT ALL

a. Life-threatening emergency
b. Abandoned (street) child with pneumonia
c. Elective cosmetic surgery
d. Child protection case of abuse

1.N8 A child with open pulmonary TB keeps attending school and parents refuse isolation. The nurse should: NCLEX

a. Respect parental choice
b. Notify the Ministry of Health and enforce isolation
c. Ignore it
d. Tell the school only

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Ethics	Science of principles of right and wrong conduct.	**Code of ethics**	Guidelines for nurse behavior and framework for ethical decisions.
Autonomy	The person's independence/self-determination.	**Beneficence**	To do good; positive action to help.
Non-maleficence	Do no harm.	**Justice**	Equal distribution of care according to priority and need.

Medicolegal dilemma	Decision that must be clinically sound, legally defensible and ethically justified — child's best interest first.
Child Law 12/1996	Marriage under 18 illegal; denial of healthcare = abuse; mandates child protection.
FGM Law 126/2008	FGM illegal; physician must refuse and may report.
Public Health Law	Isolation of infectious TB until treated.
Vaccination regulations	Polio & meningococcal mandatory for school entry.
Organ donation law	No living organ donation from children under 18.

LECTURE 2

Congenital Heart Disease

عيوب القلب الخلقية

Part A · Past Exam Questions

أسئلة الدفعات السابقة

2.1 An appropriate nursing action related to the administration of digoxin (Lanoxin) to an infant would be: (ميدترم 2021 (أطفال نظري)

a. Monitor serum calcium
b. Checking respiratory rate
c. Repeating a dose if the child vomits
d. Give dose if heart rate is 100 beats/min

2.2 The nurse can interpret that a ventricular septal defect does not cause cyanosis because of: (ميدترم 2021 (أطفال نظري)

a. Blood shunt from left to right
b. Increased pressure in the right atrium
c. Decreased pressure in the left atrium
d. Blood shunt from right to left

2.3 Patent ductus arteriosus is an opening between the pulmonary artery and aorta. (True / False) (ميدترم 2021 (أطفال نظري)

a. True
b. False

2.4 The most important nursing intervention for a child with congenital heart disease on digoxin is: تكرر في 2 مصادر (كويز 2 – 2022 (أطفال نظري)

a. Assess heart rate
b. Assess respiratory rate
c. Assess K level in blood
d. Assess intake and output

2.5 Which of the following should the nurse expect to note as a frequent complication for a child with congenital heart disease? تكرر في 3 مصادر (كويز 2 – 2022 (أطفال نظري)

a. Seizure disorder
b. Bleeding tendencies
c. Frequent vomiting and diarrhea
d. Susceptibility to respiratory infection

2.6 Which of the following should the nurse expect to note as a frequent complication for a child with acyanotic congenital heart disease? (كويز 2 – 2024 (أطفال نظري)
تكرر في 3 مصادر

a. Susceptibility to respiratory infection
b. Regular bleeding tendencies
c. Frequent vomiting and diarrhea
d. Seizure disorder and coma

2.7 Which of the following clinical findings would indicate a toxic dose of digoxin? كويز 2 – 2024 (أطفال نظري) تكرر في 2 مصادر

a. Tachycardia and dysrhythmia
b. Headache and diarrhea
c. Bradycardia and nausea
d. Tinnitus and muscular rigidity

2.8 Which of the following is most important to be checked before digoxin administration for a 10-month-old child? كويز 2 شعبة 1 (أطفال نظري)

a. Blood pressure
b. Respiratory rate
c. Temperature
d. Apical pulse

2.9 Which of the following is a cyanotic congenital heart defect? تكرر في 2 مصادر كويز 2 شعبة 1 (أطفال نظري)

a. Atrial septal defect (ASD)
b. Ventricular septal defect (VSD)
c. Tetralogy of Fallot (TOF)
d. Patent ductus arteriosus (PDA)

2.10 Which of the following symptoms should the nurse observe in a newborn with cyanotic CHD? كويز 2 شعبة 1 (أطفال نظري)

a. Dyspnea, cyanosis, and poor feeding
b. Tachycardia, bounding pulses, and edema
c. Wheezing, cough, and rales on auscultation
d. Cold extremities, bradycardia, and normal skin color

2.11 The child who had tetralogy of Fallot, a form of congenital heart disease that includes four defects. One of them is: كويز 2 شعبة 3 (أطفال نظري)

a. Ventricular septal defect
b. Atrial septal defect
c. Pulmonary atresia
d. Aortic stenosis

2.12 One of the following nursing interventions is most appropriate before digoxin administration for an 18-month-old child: تكرر في 2 مصادر كويز 2 شعبة 3 (أطفال نظري)

a. Blood pressure
b. Respiratory rate
c. Temperature
d. Apical pulse

2.13 Which of the following is a cyanotic congenital heart defect? تكرر في 2 مصادر كويز 2 أطفال نظري

a. Ventricular septal defect (VSD)
b. Atrial septal defect (ASD)
c. Patent ductus arteriosus (PDA)
d. Tetralogy of Fallot (TOF)

2.14 A child presents with cyanosis and dyspnea during feeding or crying. Which nursing intervention is most appropriate for this "cyanotic spell"? كويز 2 أطفال نظري

a. Place the child in a knee-to-chest position
b. Place the child in a supine position
c. Encourage immediate oral feeding
d. Administer intravenous antibiotics

2.15 **Which shunt is recommended for cyanosed neonates with Tetralogy of Fallot?** أسئلة سابقة

a. Blalock-Taussig shunt
b. Glenn procedure
c. Fontan procedure
d. Norwood procedure

2.16 **Which condition is associated with a "bounding pulse"?** أسئلة سابقة

a. Aortic stenosis
b. Persistent ductus arteriosus (PDA)
c. Tetralogy of Fallot (TOF)
d. Coarctation of the aorta

2.17 **What is the primary reason for closing PDA in infants?** أسئلة سابقة

a. To prevent arrhythmias
b. To reduce pulmonary vascular disease risk
c. To improve cyanosis
d. To prevent cardiac hypertrophy

2.18 **Which congenital defect is most often detected antenatally during routine fetal scans?** أسئلة سابقة

a. Ventricular septal defect (VSD)
b. Atrial septal defect (ASD)
c. Hypoplastic left heart syndrome (HLHS)
d. Pulmonary stenosis

2.19 **What is the usual timing of treatment for atrial septal defect (ASD)?** أسئلة سابقة

a. Birth
b. 6 months
c. 3-5 years
d. 10 years

2.20 **Which condition involves parallel circulations in the absence of mixing?** أسئلة سابقة

a. Tetralogy of Fallot
b. Transposition of the great arteries (TGA)
c. Coarctation of the aorta
d. Aortic stenosis

2.21 **What is the common presenting feature of coarctation of the aorta in neonates?** أسئلة سابقة

a. Continuous murmur
b. Cyanosis
c. Circulatory collapse after ductal closure
d. Bounding pulses

2.22 **Which medication is critical in managing Cyanotic congenital heart defects?** أسئلة سابقة

a. Furosemide
b. Digoxin
c. Prostaglandin E1
d. Adenosine

2.23 **What is a key auscultatory finding in pulmonary stenosis?** أسئلة سابقة

a. Continuous murmur at the back
b. Ejection systolic murmur with a click
c. Diastolic murmur at the apex
d. Holosystolic murmur

2.24 What is the definitive management for Tetralogy of Fallot? أسئلة سابقة

a. Medical treatment only
b. Cardiac catheterization
c. Surgical correction
d. Balloon valvuloplasty

2.25 Which of the following is a cyanotic heart defect? أسئلة سابقة

a. Ventricular septal defect (VSD)
b. Tetralogy of Fallot (TOF)
c. Atrial septal defect (ASD)
d. Persistent ductus arteriosus (PDA)

2.26 What does a "blue baby" most likely indicate? أسئلة سابقة

a. Acyanotic heart disease
b. Cyanotic heart disease
c. Pulmonary hypertension
d. Left-to-right shunt

2.27 Which condition involves a left-to-right shunt? أسئلة سابقة

a. Tetralogy of Fallot
b. Transposition of the great arteries
c. Atrial septal defect
d. Hypoplastic left heart syndrome

2.28 Which defect commonly leads to Eisenmenger syndrome if untreated? أسئلة سابقة

a. Pulmonary stenosis
b. Ventricular septal defect (VSD)
c. Aortic stenosis
d. Coarctation of the aorta

2.29 Which feature is typically absent in adult coarctation of the aorta? أسئلة سابقة

a. Systemic hypertension in the arms
b. Radio-femoral delay
c. Cyanosis at birth
d. Collateral vessel murmurs

2.30 Which diagnostic tool is essential for detecting congenital heart disease? أسئلة سابقة

a. ECG
b. Echocardiography
c. MRI
d. Chest X-ray

2.31 What is a characteristic feature of hypoplastic left heart syndrome? أسئلة سابقة

a. Normal left ventricle size
b. Small left-sided structures
c. Overriding aorta
d. Large ventricular septal defect

2.32 What is the primary concern in managing transposition of the great arteries? أسئلة سابقة

a. Preventing cyanosis
b. Maintaining ductal patency
c. Avoiding arrhythmias
d. Reducing pulmonary pressure

2.33 What is the most appropriate initial step for a cyanosed neonate? أسئلة سابقة

a. Start antibiotics
b. Administer prostaglandin E1
c. Perform balloon atrial septostomy
d. Conduct echocardiography

2.34 What is the primary cause of hypercyanotic spells in TOF? أسئلة سابقة

a. Right ventricular outflow tract obstruction
b. Left-to-right shunting
c. Ductal closure
d. Pulmonary hypertension

2.35 Which is the preferred treatment for pulmonary stenosis with a pressure gradient >64 mmHg? أسئلة سابقة

a. Surgical repair
b. Balloon dilatation
c. Diuretic therapy
d. Stent placement

2.36 Which congenital defect is commonly associated with Down syndrome? أسئلة سابقة

a. Tetralogy of Fallot
b. Atrioventricular septal defect (AVSD)
c. Transposition of the great arteries
d. Pulmonary stenosis

2.37 One of the following nursing interventions is most appropriate before digoxin administration is to check for 18 months old child: تكرر في 2 مصادر أسئلة سابقة

a. Blood pressure.
b. Respiratory rate.
c. Temperature.
d. Apical pulse.

2.38 Patent ductus arteriosus is an opening between the pulmonary artery and the aorta. (True / False) أسئلة سابقة

a. True
b. False

2.39 A child with tetralogy of Fallot (a congenital heart disease that includes four defects). One of them is: أسئلة سابقة

a. Ventricular septal defect
b. Atrial septal defect
c. Pulmonary atresia
d. Aortic stenosis

2.40 The most important nursing intervention for a child with congenital heart disease and on digoxin is: تكرر في 2 مصادر أسئلة سابقة

a. Assess heart rate.
b. Assess respiratory rate.
c. Assess K level in blood.
d. Assess intake and output.

2.41 Before giving Digoxin to an infant, the heart rate should not be less than: أسئلة سابقة

a. 100 beat/min
b. 60 beat / min.
c. 80 beat / min.
d. 120 beat / min.

2.42 Which of the following should the nurse expect to note as a frequent complication for a child with a cyanotic congenital heart disease? تكرر في 3 مصادر أسئلة سابقة

a. Susceptibility to respiratory infection
b. Regular bleeding tendencies
c. Frequent vomiting and diarrhea
d. Seizure disorder and coma

2.43 Which of the following clinical findings would indicate a toxic dose of digoxin? أسئلة سابقة

تكرر في 2 مصادر

a. Tachycardia and dysrhythmia
b. Headache and diarrhea
c. Bradycardia and nausea
d. Tinnitus and muscular rigidity

2.44 The nurse is to administer Digoxin Elixir to a 6-month-old with a congenital heart defect. The nurse auscultates an apical pulse rate of 100. the nurse should: أسئلة سابقة

a. Record the heart rate and call the physician
b. Record the heart rate and administer the medication
c. Administer the medication and recheck the heart rate in 30 minutes
d. Hold the medication and recheck the heart rate in 30 minutes.

2.45 Which of the following can indicate left-sided heart failure in an infant? أسئلة سابقة

a. Fever
b. Low appetite
c. Increased respiratory rate
d. Crying

2.46 Congenital anomaly is a structural defect present at birth? أسئلة سابقة

a. Ture
b. False

2.47 Cyanotic CHD is: أسئلة سابقة

a. PDA
b. VSD
c. TGA
d. ASD

2.48 A 5-year-old with congestive heart failure has been receiving Digoxin (Lanoxin). Which finding indicated that the medication is having a desired effect. أسئلة سابقة

a. Increased urinary output
b. Stabilized weight
c. Improved appetite
d. Increased pedal edema

2.49 A nurse is caring for an infant recently diagnosed with a congenital heart defect. Which clinical sign would most likely be present? أسئلة سابقة

a. Slow pulse rate
b. Weight gain
c. Decreased systolic pressure
d. Irregular WBC lab values

2.50 Foramen oval An opening between two atrium? أسئلة سابقة

a. True
b. False

2.51 Signs of congestive heart failure is : أسئلة سابقة

a. Edema
b. Cyanosis
c. Anorexia
d. All of Above

2.52 Condition in which heart cannot pump adequate blood to tissues: أسئلة سابقة

a. Rheumatic fever
b. Cardiomyopathy
c. Pericarditis
d. Congestive heart failure

أسئلة سابقة

2.53 Persistence of a fetal duct between the pulmonary artery and the aorta.

a. Patent dictus arteriosus
b. Circulation of aorta
c. Ventricular septal defect
d. Atrial septal defect

أسئلة سابقة

2.54 The earliest sign of congestive heart failure on a chest X-ray is

a. Increased heart size
b. Kerley B lines
c. Central pulmonary vascular
d. Pleural effusion

أسئلة سابقة

2.55 Management of TGA including:

a. Prostaglandin E1
b. Indomethacin
c. Ibuprofen
d. All of Above

أسئلة سابقة

2.56 Clinical manifestation of cyanotic CHD:

a. Anoxic spells
b. Systolic murmur
c. Diaphoresis
d. Tachycardia

أسئلة سابقة

2.57 An infant with a marked cyanotic congenital heart defect with decreased pulmonary vascularity should be treated with:

a. Digoxin
b. Indomethacin
c. Prostaglandin E1
d. Epinephrine

أسئلة سابقة

2.58 Corrective surgery in management of tetralogy of Fallot done by 1 year?

a. True
b. False

أسئلة سابقة

2.59 Therapy of blue spell in Fallot tetralogy includes all the following, Except

a. Knee chest position
b. Oxygen
c. Morphine
d. Dopamine

أسئلة سابقة

2.60 What is the most common complication of Infective endocarditis?

a. Hematuria.
b. Pericardial effusion.
c. Splenomegaly.
d. Cerebral stroke
e. CHF

أسئلة سابقة

2.61 All of the following are risk factors for developing CHD, EXCEPT:

a. Diabetes mellitus and rubella
b. Alcohol consumption
c. Teratogenic drugs such as anticonvulsants
d. Younger maternal age
e. Secondhand tobacco smoke
f. Increased maternal age

أسئلة سابقة

2.62 Cyanotic congenital heart defect is seen in TGA with increased pulmonary blood flow. (True / False)

a. True
b. False

2.63 Congenital heart defect may be a component of down syndrome

أسئلة سابقة

a. True **b.** False

2.64 Acyanotic congenital heart defects are seen in all of the following, EXCEPT:

أسئلة سابقة

a. Aortic stenosis **b.** Pulmonic stenosis
c. Tetralogy of Fallot **d.** Atrial septal defect

2.65 ASD is an abnormal opening between the atria allowing blood to flow from the right atrium to the left atrium. (True / False)

أسئلة سابقة

a. True **b.** False

2.66 Patent Ductus Arteriosus (PDA) is

أسئلة سابقة

a. Failure of the fetal ductus arteriosus to close within the first week of life.
b. Results in right-to-left shift of blood flow from pulmonary artery to the aorta.
c. Results in left-to-right shift of blood flow from aorta to the pulmonary artery.
d. A and B
e. A and C

2.67 Ventricular Septal Defect (VSD) is

أسئلة سابقة

a. Abnormal opening between the Rt. And Lt.Ventricles.
b. Either membranous (20%) or muscular (80%).
c. More than 75% of the cases may close spontaneously.
d. All of the above

2.68 All of the following are clinical manifestations of acyanotic congenital heart disease EXCEPT:

أسئلة سابقة

a. Dyspnea **b.** Diaphoresis
c. Bradypnea **d.** Systolic murmur

2.69 For mild ASD the appropriate management is surgical closure at 4–5 years. (True / False)

أسئلة سابقة

a. True **b.** False

2.70 Pulmonary blood flow is increased by placing a band around the pulmonary artery as palliative management for VSD. (True / False)

أسئلة سابقة

a. True **b.** False

2.71 Complete surgical repair for VSD depends on the presence of pulmonary hypertension and can be performed from one year to school age

أسئلة سابقة

a. True **b.** False

2.72 For a premature infant of PDA the medical treatment can be all of the following except:

أسئلة سابقة

a. Indomethacin **b.** Ibuprofen
c. Prostaglandin E1 **d.** A and C

2.73 Surgical correction of PDA is done by ligation via right thoracotomy with cardiopulmonary bypass. (True / False) أسئلة سابقة

a. True **b.** False

2.74 Transportation of great arteries (TGA) is: أسئلة سابقة

a. Abnormal opening between the Rt. And Lt Ventricles.
b. Failure of the fetal ductus arteriosus to close
c. Abnormal opening between the atria allow blood to flow from left atrium to right atrium.
d. Aorta originating from the right ventricle, and pulmonary originating from the left ventricle.

2.75 As a management for TGA, all of the following can be performed except: أسئلة سابقة

a. Ballon Atrial septostomy **b.** Arterial switch procedure
c. Left thoracicotomy **d.** Prostaglandin E1 infusion

2.76 In TGA it is essential to keep the PDA closed, by administering prostaglandin E1 infusion. (True / False) أسئلة سابقة

a. True **b.** False

2.77 Tetralogy Of Fallot (TOF) is the presence of four main defects, choose the wrong one: أسئلة سابقة

a. Pulmonary stenosis **b.** Overriding aorta
c. Atrial septal defect (ASD) **d.** Right ventricle hypertrophy

2.78 An infant presented with all the following manifestations: metabolic acidosis, poor growth, exercise intolerance, clubbing fingers, polycythemia and anoxic spells, the nurse expect the infant is experiencing: أسئلة سابقة

a. Atrial septal defect (ASD) **b.** Ventricular septal defect (VSD)
c. A Cyanotic CHD **d.** Cyanotic CHD

2.79 Restless attacks may follow feeding or defecation as a result of low O2 levels in the blood cause the lips, fingers, and toes to look blue, and lead to unconsciousness, convulsion, and death is which of the following: أسئلة سابقة

a. Clubbing fingers **b.** Polycythemia
c. Anoxic spells **d.** None of the above

2.80 For treating hypercyanotic spells, all of the following can be done EXCEPT: أسئلة سابقة

a. Place infant in knee-chest (squatting) position
b. Employ a calm, comforting approach
c. Administer 100% oxygen by face mask
d. Give morphine subcutaneously or IV
e. Place infant in prone position
f. Begin IV fluid replacement and volume expansion if needed

أسئلة سابقة

2.81 **Management of Tetralogy of Fallot are which of the following**

a. Prophylactic antibiotics
b. Palliative shunt
c. Corrective surgery
d. All of the above

أسئلة سابقة

2.82 **Which of the following can be a long-term complication of TOF?**

a. Cerebral thrombosis
b. Viral endocarditis
c. Urinary tract infection
d. Renal failure

أسئلة سابقة

2.83 **The aim of performing a palliative shunt in TOF (Tetralogy of Fallot) is to**

a. Increase aortic artery blood flow.
b. Decrease pulmonary artery blood flow.
c. Increase pulmonary artery blood flow.
d. Decrease aortic artery blood flow.

أسئلة سابقة

2.84 **Is the inability of the heart to pump an adequate amount of blood to the systemic circulation to meet the metabolic demands of the body.**

a. ASD
b. TGA
c. TOF
d. CHF

أسئلة سابقة

2.85 **In Right-sided heart failure the following are seen except:**

a. Increased pressure in right atrium
b. Decreased systemic venous circulation
c. Hepato-splenomegaly
d. Edema

أسئلة سابقة

2.86 **In Left-sided heart failure which of the following can be seen:**

a. Increased Pressure in left atrium
b. Increased pressure in pulmonary veins
c. Pulmonary edema
d. All of the above

أسئلة سابقة

2.87 **The inability of the heart to maintain an adequate cardiac output to kidneys (myocardial failure) lead to:**

a. Increased sodium and water reabsorption
b. Fluid overload (increase workload on heart).
c. Congestion in pulmonary and systemic circulation
d. All of the above

أسئلة سابقة

2.88 **An infant presented with Tachypnea, Wheezing, Crackles, Retractions, Dyspnea, Orthopnea cough, grunting, flaring nares, irritability, tiring with feeding, hoarseness, and cyanosis. The nurse expected that the infant developed**

a. Systemic Venous Congestion
b. Atrial septal defect
c. Ventricular septal defect
d. Pulmonary Venous Congestion

أسئلة سابقة

2.89 **Giving the following manifestations for a child: Hepatomegaly, Ascites, Peripheral Edema and Neck Vein Distention, the nurse can expect that the child is experiencing which of the following:**

a. Ventricular septal defect
b. Pulmonary Venous Congestion
c. Systemic Venous Congestion
d. Atrial septal defect

2.90 Which of the following can be a clinical manifestation for Impaired myocardial function: أسئلة سابقة

a. Bradycardia and Sweating.
b. Weak Peripheral Pulses and hypertension.
c. Capillary refill time less than 2 seconds.
d. Decreased urine output (oliguria).

2.91 Therapy of CHF includes the following EXCEPT: أسئلة سابقة

a. Digoxin
b. Captopril (Capoten)
c. O2 therapy
d. Propranolol
e. Furosemide (Lasix)

2.92 Mr. and Mrs. Baker's only daughter is diagnosed with heart failure. Which of the following interventions would be appropriate to promote optimal nutrition for the infant? أسئلة سابقة

a. Replacing regular nipples with easy-to-suck ones
b. Allowing the infant to feed for at least 1 hour
c. Providing large feedings evenly spaced every 4 hours
d. Offering formula that is high in sodium and calories

2.93 The main action of Digoxin is all the following except: أسئلة سابقة

a. Increase cardiac contractility
b. Increase cardiac output
c. Increase heart rate
d. A and C

2.94 The following can be side effects of digoxin EXCEPT: أسئلة سابقة

a. Dysrhythmias
b. Anorexia
c. Visual disturbances
d. Tachycardia
e. Nausea and vomiting

2.95 Before administration of Digoxin the nurse should do which of the following: أسئلة سابقة

a. Take the apical pulse for 1 minute.
b. Give digoxin at regular intervals.
c. Administer the drugs carefully by slowly.
d. Not mix it with other foods or fluids.
e. All of the above.

2.96 For a child who receives Digoxin, he should be given water after administering the drug or brush the teeth أسئلة سابقة

a. True
b. False

2.97 Which of the following is a cyanotic congenital heart disease? أسئلة سابقة

a. Atrial septal defect (ASD)
b. Ventricular septal defect (VSD)
c. Tetralogy of Fallot (TOF)
d. Patent ductus arteriosus (PDA)

2.98 What is the most common congenital heart defect? أسئلة سابقة

a. Coarctation of the aorta (CoA)
b. Atrial septal defect (ASD)
c. Ventricular septal defect (VSD)
d. Transposition of the great arteries (TGA)

2.99 What is the key pathophysiological feature of right-to-left shunt defects? أسئلة سابقة

a. Pulmonary congestion
b. Decreased systemic oxygenation
c. Increased pulmonary blood flow
d. Hypertrophy of the left ventricle

2.100 Which diagnostic test is considered the gold standard for detecting congenital heart disease? أسئلة سابقة

a. Electrocardiogram (ECG)
b. Chest X-ray
c. Echocardiography (ECHO)
d. Pulse oximetry

2.101 Which maternal condition increases the risk of congenital heart disease in the fetus? أسئلة سابقة

a. Hypothyroidism
b. Hyperlipidemia
c. Diabetes
d. Migraine

2.102 What structural defect is present in transposition of the great arteries (TGA)? أسئلة سابقة

a. A single artery replaces the aorta and pulmonary artery
b. The aorta and pulmonary artery are switched
c. A missing tricuspid valve
d. A narrowed aortic valve

2.103 What is a key clinical feature of coarctation of the aorta (CoA)? أسئلة سابقة

a. Bounding pulses in the lower extremities
b. Murmur heard best at the apex
c. Weak femoral pulses and hypertension in upper limbs
d. Cyanosis in all extremities

2.104 Which treatment keeps the ductus arteriosus open in ductal-dependent heart defects? أسئلة سابقة

a. Diuretics
b. ACE inhibitors
c. Prostaglandin E1
d. Beta-blockers

2.105 What is the primary reason for polycythemia in cyanotic congenital heart disease? أسئلة سابقة

a. Increased pulmonary congestion
b. Chronic hypoxia leading to increased erythropoiesis
c. Reduced cardiac output
d. Left ventricular hypertrophy

2.106 Which preventive measure helps reduce the risk of congenital heart disease? أسئلة سابقة

a. High-protein diet during pregnancy
b. Early childhood vaccinations
c. Avoiding teratogenic medications
d. Daily aspirin during pregnancy

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

2.N1 Which defect produces a left-to-right shunt and is acyanotic? NCLEX

a. Tetralogy of Fallot
b. Transposition of great arteries
c. Ventricular septal defect
d. Tricuspid atresia

2.N2 The four defects of tetralogy of Fallot are: NCLEX

a. ASD, VSD, PDA, coarctation
b. Pulmonary stenosis, VSD, overriding aorta, right ventricular hypertrophy
c. Aortic stenosis, ASD, LVH, PDA
d. Mitral stenosis, VSD, ASD, PDA

2.N3 Select ALL nursing actions for a hypercyanotic (Tet) spell. SELECT ALL

a. Knee-chest position
b. Calm, comforting approach
c. 100% oxygen
d. Morphine as prescribed
e. Place supine flat and leave alone

2.N4 Before giving digoxin to an infant, the nurse counts the apical pulse for: NCLEX

a. 15 seconds
b. 30 seconds
c. 1 full minute
d. 2 minutes

2.N5 Which drug keeps the ductus arteriosus open in ductal-dependent lesions such as TGA? NCLEX

a. Indomethacin
b. Ibuprofen
c. Prostaglandin E1
d. Digoxin

2.N6 An infant with CHF has poor feeding. Which intervention promotes nutrition? NCLEX

a. Large feedings every 4 hours
b. Small frequent feedings with soft easy-to-suck nipples
c. Feed for at least 1 hour
d. High-sodium formula

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Acyanotic (L→R)	VSD, ASD, PDA — increased pulmonary blood flow.	**Obstructive**	Coarctation, aortic stenosis, pulmonary stenosis.
Cyanotic (R→L)	TOF, TGA, tricuspid atresia.	**Most common CHD**	VSD.
TOF	PS + VSD + overriding aorta + RVH.	**Tet spell care**	Knee-chest, calm, 100% O2, morphine, IV fluids.

TGA	Aorta from RV, pulmonary artery from LV; needs mixing — PGE1, balloon septostomy, arterial switch.
Coarctation	Narrowed aorta; high BP arms, weak femoral pulses, radio-femoral delay.
CHF in infant	Tachypnea, tiring with feeds, sweating, hepatomegaly, weight gain.

PDA	Duct between aorta and pulmonary artery; bounding pulse, machinery murmur; indomethacin/ibuprofen in preterm.
Digoxin	Count apical pulse 1 min; toxicity: bradycardia, nausea, vomiting, visual changes.
Gold standard diagnosis	Echocardiography.

LECTURE 3

Physical Assessment of the Child

الفحص الإكلينيكي للطفل

Part A · Past Exam Questions

أسئلة الدفعات السابقة

3.1 **Which finding in a 3-year-old would be abnormal and require further evaluation?** أسئلة سابقة

a. Using three-word sentences
b. Fear of separation from parents during the exam
c. Inability to jump or stand on one foot
d. Pretend play and using toys interactively

3.2 **Which respiratory sign would indicate a concern when documenting a child's respiratory system?** أسئلة سابقة

a. Normal breath sounds with no wheezing
b. Use of accessory muscles for breathing
c. Clear nasal passages
d. Regular and even breathing rate

3.3 **A 4-week-old has a fever of 38.5 °C and is irritable with no localizing signs. What is the best intervention?** أسئلة سابقة

a. Discharge home with antipyretics
b. Blood culture, urine culture, LP and admit for IV antibiotics
c. Oral antibiotics and follow-up in 24 hours
d. Chest X-ray only

3.4 **Which approach is most effective when explaining abnormal assessment findings to parents?** أسئلة سابقة

a. Detailed medical explanations without considering emotions
b. Compassionate tone, simple terms and offering next steps
c. Minimizing the significance of findings
d. Directing parents to medical literature

3.5 **A 2-month-old presents with floppy posture, poor head control and hypotonia. Which is the most concerning possible cause?** أسئلة سابقة

a. Cerebral palsy
b. Anemia
c. Muscular dystrophy
d. Spinal muscular atrophy

3.6 **A baby makes broad arm movements before learning to pick up small objects with the fingers. This best illustrates which principle?** أسئلة سابقة

a. Principle of continuity
b. Principle of cephalocaudal development
c. Principle of integration
d. Principle of predictability

3.7 **Which choice includes the components of a complete pediatric history?** أسئلة سابقة

a. Statistical information, client profile, health history, family history, review of systems, lifestyle and life patterns
b. Vital signs, chief complaint and list of previous problems
c. Chief complaint with location, quality, quantity, timeframe and modifying factors
d. Pertinent developmental and family information

3.8 **What is a common challenge in pediatric assessment due to differences in beliefs about medical care?** أسئلة سابقة

a. Fear and non-cooperation
b. Cultural barriers
c. Developmental variations
d. Communication difficulties

3.9 **What is the significance of a bulging fontanelle in an infant during a physical examination?** أسئلة سابقة

a. Dehydration
b. Increased intracranial pressure (ICP)
c. Normal finding
d. Infection

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

3.N1 **The correct order of abdominal examination in a child is:** NCLEX

a. Palpation, percussion, auscultation, inspection
b. Inspection, auscultation, percussion, palpation
c. Percussion, palpation, inspection, auscultation
d. Auscultation, inspection, palpation, percussion

3.N2 **Symptoms reported by the mother are considered:** NCLEX

a. Objective data
b. Subjective data
c. Diagnostic data
d. Laboratory data

3.N3 **Select ALL features of Down syndrome facies.** SELECT ALL

a. Upslanting palpebral fissures
b. Flattened nasal bridge
c. Brushfield spots
d. Epicanthal fold
e. Frontal bossing from hemolysis

3.N4 **A bulging anterior fontanel in a quiet infant suggests:** NCLEX

a. Dehydration
b. Increased intracranial pressure
c. Normal finding
d. Malnutrition

3.N5 **Before palpating a child's abdomen the nurse should:** NCLEX

a. Use cold hands to stimulate
b. Warm the hands, position supine and talk to relax the child
c. Ask the child to sit and cough
d. Palpate the painful area first

3.N6 Chipmunk facies is characteristic of: NCLEX

a. Rickets
b. Chronic hemolytic anemia (thalassemia)
c. Adenoid hypertrophy
d. Hypothyroidism

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Subjective data	Symptoms told by patient/mother.	**Objective data**	Signs found by examiner.
Inspection	Visual observation: mental status, color, movement, nutrition, symmetry.	**Palpation**	Feeling with warm hands; supine; talk to relax.
Percussion	Tapping; notes dull, resonant, tympanitic.	**Auscultation**	Listening with stethoscope.
Abdomen order	Inspection → auscultation → percussion → palpation.	**Pallor**	Anemia or shock.
Bulging fontanel	Raised ICP.	**Sunken fontanel**	Dehydration.
Down facies	Upslanting eyes, flat nasal bridge, Brushfield spots, epicanthal fold, low-set ears.	**Rickets signs**	Delayed fontanel closure, frontal bossing, craniotabes, rosary, Harrison's sulcus.

LECTURE 4

Respiratory Disorders in Children

أمراض الجهاز التنفسي عند الأطفال

Part A · Past Exam Questions

أسئلة الدفعات السابقة

4.1 **Which is the specific nursing diagnosis for a child with tonsillitis?** ميدترم 2020 (أطفال نظري)

a. Anxiety/fear related to unfamiliar event
b. Altered family process related to hospitalization
c. Impaired swallowing related to inflammation
d. High risk for injury

4.2 **The primary therapeutic regimen for croup syndrome usually includes:** ميدترم 2021 (أطفال نظري)

a. Intubation, racemic epinephrine, and corticosteroids
b. Intubation, racemic epinephrine, and antibiotics
c. Vigilant assessment, racemic epinephrine, and antibiotics
d. Vigilant assessment, racemic epinephrine, and corticosteroids

4.3 **In the postoperative period following a tonsillectomy, the child should be:** ميدترم 2021 (أطفال نظري)

a. Encouraged to cough and deep breathe
b. Observed for signs of hemorrhage
c. Suctioned vigorously to clear the airway
d. Placed in Trendelenburg position

4.4 **What is the most important goal for a child with ineffective airway clearance?** ميدترم 2021 (أطفال نظري)

a. Providing adequate oral fluids
b. Reducing the child's anxiety
c. Mobilizing/clearing the child's secretions
d. Administering medication as ordered

4.5 **What is a common symptom of upper respiratory tract infections?** أسئلة سابقة

a. Wheezing
b. Nasal congestion
c. Cyanosis
d. Clubbing

4.6 **Which microorganism is commonly associated with pneumonia in children?** أسئلة سابقة

a. Streptococcus pneumoniae
b. Escherichia col
c. Candida albicans
d. Human papillomavirus

4.7 **What is the mainstay treatment for viral respiratory infections?** أسئلة سابقة

a. Antibiotics
b. Supportive care
c. Antifungals
d. Bronchodilators

4.8 What is the typical sound heard during asthma exacerbation? أسئلة سابقة

a. Crackles
b. Wheezing
c. Stridor
d. Silent chest

4.9 Which of the following is a risk factor for respiratory disorders in children? أسئلة سابقة

a. Vaccination
b. Passive smoking
c. Breastfeeding
d. Exercise

4.10 Which investigation is preferred to confirm pneumonia? أسئلة سابقة

a. Chest X-ray
b. ECG
c. Spirometry
d. Skin prick test

4.11 Which drug is used as a first-line treatment for acute bronchial asthma? أسئلة سابقة

a. Amoxicillin
b. Salbutamol
c. Prednisolone
d. Hydrocortisone

4.12 What is the most likely diagnosis in a child with barking cough and inspiratory stridor? أسئلة سابقة

a. Asthma
b. Croup
c. Pneumonia
d. Bronchitis

4.13 Which vaccine helps prevent certain respiratory infections? أسئلة سابقة

a. BCGB
b. Influenza vaccine
c. Hepatitis B vaccine
d. MMR

4.14 What is the typical clinical feature of bronchiolitis? أسئلة سابقة

a. Productive cough
b. Fever and cyanosis
c. Wheezing and respiratory distress
d. Pleural effusion

4.15 Which of the following conditions is considered a lower respiratory tract disorder? أسئلة سابقة

a. Pharyngitis
b. Tonsillitis
c. Bronchitis
d. Sinusitis

4.16 Which environmental factor is most harmful for children with asthma? أسئلة سابقة

a. High humidity
b. Polluted air
c. Cold temperature
d. Dry air

4.17 What is the hallmark of severe respiratory distress in infants? أسئلة سابقة

a. Crying loudly
b. Nasal flaring
c. Excessive sleep
d. Normal breathing rate

4.18 Which treatment is NOT suitable for viral bronchiolitis? أسئلة سابقة

a. Antibiotics
b. Oxygen therapy
c. Hydration
d. Supportive care

4.19 What is the most common pathogen causing bronchiolitis? أسئلة سابقة

a. RSV (Respiratory Syncytial Virus)
b. Adenovirus
c. Influenza virus
d. Mycoplasma pneumoniae

4.20 Which of the following is a complication of severe pneumonia? أسئلة سابقة

a. Otitis media
b. Pleural effusion
c. Nasal polyps
d. Conjunctivitis

4.21 What is the primary purpose of bronchodilators in asthma management? أسئلة سابقة

a. Treat infection
b. Relax airway muscles
c. Reduce fever
d. Increase oxygen absorption

4.22 Which symptom indicates respiratory failure in children? أسئلة سابقة

a. Tachycardia
b. Cyanosis
c. Dry cough
d. Mild fever

4.23 Which physical sign is most commonly associated with croup? أسئلة سابقة

a. Productive cough
b. Barking cough
c. Inspiratory crackles
d. Expiratory wheezing

4.24 What is the most appropriate initial management for mild croup? أسئلة سابقة

a. Intravenous antibiotics
b. Supportive care with humidified air
c. Nebulized bronchodilators
d. Mechanical ventilation

4.25 What is the hallmark symptom of foreign body aspiration in children? أسئلة سابقة

a. Sudden onset of coughing or choking
b. Chronic productive cough
c. High fever
d. Nasal congestion

4.26 Which investigation is most useful in diagnosing foreign body aspiration? أسئلة سابقة

a. Chest X-ray
b. CT scan
c. Bronchoscopy
d. Blood culture

4.27 Which of the following is a feature of severe asthma exacerbation? أسئلة سابقة

a. Silent chest
b. Increased appetite
c. Mild wheezing
d. No use of accessory muscles

4.28 Which age group is most commonly affected by bronchitis? أسئلة سابقة

a. Neonates (0–1 month)
b. Infants (2–12 months)
c. Toddlers (1–3 years)
d. School-aged children

4.29 What is the typical finding in chest auscultation of a child with pneumonia? أسئلة سابقة

a. Stridor
b. Wheezing
c. Crackles
d. Clear lung fields

أسئلة سابقة

4.30 Which factor significantly increases the risk of respiratory infections in infants?

a. Breastfeeding
b. Malnutrition
c. Regular vaccination
d. Playing outdoors

أسئلة سابقة

4.31 Most common cause of bronchiolitis in infants:

a. Influenza
b. RSV
c. Adenovirus
d. Rhinovirus

أسئلة سابقة

4.32 Hallmark symptom of croup:

a. Productive cough
b. Hemoptysis
c. High fever and chest pain
d. Stridor and barking cough

أسئلة سابقة

4.33 A 2-year-old with sudden cough, wheezing and decreased breath sounds on the right most likely has:

a. Asthma
b. Foreign body aspiration
c. Pneumonia
d. Bronchiolitis

أسئلة سابقة

4.34 Common cause of bacterial pneumonia in children under 5:

a. Mycoplasma
b. Streptococcus pneumoniae
c. Chlamydia trachomatis
d. Staphylococcus aureus

أسئلة سابقة

4.35 First-line treatment for mild asthma in children:

a. Inhaled SABA
b. Leukotriene antagonists
c. Oral steroids
d. Inhaled steroids

أسئلة سابقة

4.36 Most common bacterial cause of acute tonsillitis:

a. H. influenzae
b. S. pneumoniae
c. Group A Streptococcus (S. pyogenes)
d. S. aureus

أسئلة سابقة

4.37 Investigation to confirm pneumonia:

a. CBC
b. Chest X-ray
c. Blood culture
d. Pulse oximetry

أسئلة سابقة

4.38 Sign of severe respiratory distress in a child:

a. Mild nasal flaring
b. Occasional cough
c. Grunting respiration
d. Sneezing

أسئلة سابقة

4.39 Primary mechanism of obstructive sleep apnea in obesity:

a. Decreased surfactant
b. Allergic inflammation
c. Mechanical upper-airway obstruction during sleep
d. Impaired alveolar gas exchange

4.40 Key diagnostic feature of asthma in children: أسئلة سابقة

a. Persistent productive cough
b. Clubbing
c. Reversible airway obstruction
d. Unilateral wheezing

4.41 The immediate nursing care for a child admitted to the hospital with pneumonia should be: أسئلة سابقة

a. High-protein diet
b. Chest physiotherapy
c. Measure temperature
d. Bed rest

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

4.N1 A child has a barking cough and inspiratory stridor. The likely diagnosis is: NCLEX

a. Asthma
b. Croup (laryngotracheobronchitis)
c. Pneumonia
d. Bronchiolitis

4.N2 The most common cause of bronchiolitis is: NCLEX

a. Adenovirus
b. RSV
c. Streptococcus
d. Mycoplasma

4.N3 Select ALL signs of respiratory distress in an infant. SELECT ALL

a. Nasal flaring
b. Intercostal retractions
c. Grunting
d. Tachypnea
e. Bradypnea with calm sleep

4.N4 A 'silent chest' in an asthmatic child indicates: NCLEX

a. Improvement
b. Severe life-threatening exacerbation
c. Mild attack
d. Normal finding

4.N5 After tonsillectomy the nurse watches especially for: NCLEX

a. Frequent swallowing (bleeding)
b. Mild sore throat
c. Low-grade fever
d. Thirst

4.N6 First-line reliever in acute asthma is: NCLEX

a. Inhaled salbutamol
b. Amoxicillin
c. Montelukast daily
d. Antihistamine

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

URTI	Common cold, pharyngitis, tonsillitis, sinusitis, otitis.	**LRTI**	Bronchitis, bronchiolitis, pneumonia.
Croup	Barking cough, inspiratory stridor; humidified air, steroids, racemic epinephrine.	**Epiglottitis**	Drooling, tripod; don't examine throat.

Bronchiolitis	RSV; wheeze + distress in infants; supportive care.
Asthma	Wheeze; salbutamol reliever; avoid smoke/pollution.
Foreign body	Sudden choking; bronchoscopy.
Respiratory failure	Cyanosis, altered consciousness.

Pneumonia	Fever, tachypnea, crackles; CXR; S. pneumoniae.
Status asthmaticus	Silent chest — emergency.
Tonsillectomy	Watch frequent swallowing = bleeding.
Risk factors	Passive smoking, malnutrition, no vaccination.

LECTURE 5

Rheumatic Fever

الحمى الروماتيزمية

Part A · Past Exam Questions

أسئلة الدفعات السابقة

5.1 What is the primary definition of Rheumatic Fever? أسئلة سابقة

a. A viral infection in children
b. An inflammatory disease following a bacterial infection
c. A hereditary condition
d. An immune deficiency disorder

5.2 Which of the following is the main causative agent of Rheumatic Fever? أسئلة سابقة

a. Streptococcus pyogenes
b. Escherichia coli
c. Staphylococcus aureus
d. Clostridium difficile

5.3 What are the most common symptoms of Rheumatic Fever? أسئلة سابقة

a. Sore throat and difficulty swallowing
b. Fever, joint pain, and skin rash
c. Nausea and vomiting
d. Severe headache and dizziness

5.4 According to the Jones Criteria, which of the following is a major criterion for diagnosing Rheumatic Fever? أسئلة سابقة

a. Subcutaneous nodules
b. Fatigue
c. Cough
d. Chest pain

5.5 Which investigation is most useful for confirming a recent streptococcal infection? أسئلة سابقة

a. Throat swab culture
b. Chest X-ray
c. Echocardiography
d. Complete blood count

5.6 What is the primary goal of Rheumatic Fever treatment? أسئلة سابقة

a. Relieving symptoms only
b. Preventing recurrence and complications
c. Improving physical fitness
d. Controlling blood pressure

5.7 What is a common complication of Rheumatic Fever affecting the heart? أسئلة سابقة

a. Valvular damage
b. Myocardial infarction
c. Pericardial effusion
d. Atrial fibrillation

أسئلة سابقة

5.8 The mainstay of prevention for Rheumatic Fever is:

a. Vaccination
b. Early treatment of streptococcal infections
c. Routine physical check-ups
d. Antibiotic prophylaxis

أسئلة سابقة

5.9 Which medication is commonly used for long-term prophylaxis in Rheumatic Fever?

a. Penicillin
b. Paracetamol
c. Amoxicillin
d. Azithromycin

أسئلة سابقة

5.10 Rheumatic Fever most commonly affects which age group?

a. Infants under 1 year
b. Children aged 5-15 years
c. Adults aged 30-50 years
d. Elderly over 65 years

أسئلة سابقة

5.11 Which major organ system is primarily involved in Rheumatic Fever?

a. Nervous system
b. Cardiovascular system
c. Respiratory system
d. Digestive system

أسئلة سابقة

5.12 Which of the following is not part of the Jones Criteria?

a. Arthritis
b. Chorea
c. Skin ulcers
d. Carditis

أسئلة سابقة

5.13 What is the gold standard diagnostic test for Rheumatic Heart Disease?

a. Echocardiography
b. Electrocardiogram (ECG)
c. MRI
d. CT scan

أسئلة سابقة

5.14 Which is a minor criterion in the Jones Criteria?

a. Fever
b. Carditis
c. Chorea
d. Subcutaneous nodules

أسئلة سابقة

5.15 Which clinical feature suggests Sydenham's chorea in Rheumatic Fever?

a. Uncontrollable jerky movements
b. Severe chest pain
c. Shortness of breath
d. Muscle weakness

أسئلة سابقة

5.16 The pathogenesis of Rheumatic Fever primarily involves:

a. Autoimmune cross-reactivity
b. Direct bacterial invasion
c. Viral mutation
d. Nutritional deficiencies

أسئلة سابقة

5.17 Which antibiotic is preferred for treating streptococcal throat infections to prevent Rheumatic Fever?

a. Erythromycin
b. Penicillin
c. Ciprofloxacin
d. Metronidazole

5.18 What is the main sign of acute carditis in Rheumatic Fever? أسئلة سابقة

a. Heart murmur
b. Edema
c. Severe hypertension
d. Cyanosis

5.19 How long should antibiotic prophylaxis continue in patients with Rheumatic Fever without carditis? أسئلة سابقة

a. 1 year
b. 5 years
c. 15 years
d. Until age 21

5.20 Rheumatic fever is caused by Streptococcus pyogenes. (True / False) أسئلة سابقة

a. True
b. False

5.21 Carditis is a minor criterion in the Jones criteria. (True / False) أسئلة سابقة

a. True
b. False

5.22 Rheumatic fever primarily affects the respiratory system. (True / False) أسئلة سابقة

a. True
b. False

5.23 Antibiotic prophylaxis is unnecessary if a patient has no symptoms of rheumatic fever. (True / False) أسئلة سابقة

a. True
b. False

5.24 Subcutaneous nodules are a major criterion in the Jones criteria. (True / False) أسئلة سابقة

a. True
b. False

5.25 Symptoms of rheumatic fever typically appear how long after a streptococcal infection? أسئلة سابقة

a. 1–2 days
b. 3–5 days
c. 2–3 weeks
d. 1 month

5.26 Which joints are most commonly affected by pain or swelling in rheumatic fever? أسئلة سابقة

a. Shoulders and hips
b. Knees, ankles, elbows and wrists
c. Fingers and toes
d. Spine and neck

5.27 What type of joint pain is often seen in rheumatic fever? أسئلة سابقة

a. Persistent pain in one joint
b. Pain that moves from joint to joint (migratory)
c. Pain relieved by rest
d. Pain without swelling or redness

5.28 Which is NOT a typical manifestation of rheumatic fever? أسئلة سابقة

a. Fever
b. Joint swelling
c. Skin rash
d. Headache

5.29 Under the 2015 Modified Jones Criteria, which symptom most likely indicates rheumatic fever? أسئلة سابقة

a. Severe headache
b. Hot or tender joints
c. Abdominal pain
d. Nausea and vomiting

5.30 Which body systems are typically affected in rheumatic fever? أسئلة سابقة

a. Respiratory, digestive and immune
b. Musculoskeletal, cardiovascular and nervous
c. Digestive, endocrine and respiratory
d. Cardiovascular, digestive and reproductive

5.31 Under the 2015 Modified Jones Criteria, which is a common sign? أسئلة سابقة

a. Skin lesions
b. Fever and joint pain
c. Gastrointestinal distress
d. Shortness of breath

5.32 Which is the characteristic skin manifestation of rheumatic fever? أسئلة سابقة

a. Erythema nodosum
b. Erythema marginatum
c. Psoriasis
d. Vitiligo

5.33 What is the primary treatment of rheumatic fever (eradicating the streptococcus)? أسئلة سابقة

a. Corticosteroids
b. Aspirin
c. Penicillin
d. Sulfadiazine

5.34 Which is NOT a sign or symptom of rheumatic fever? أسئلة سابقة

a. Joint pain
b. Fatigue
c. Chest pain
d. Nausea

5.35 One minor manifestation for rheumatic fever is أسئلة سابقة

a. Erythema marginatum
b. Poly arthritis and chorea
c. Increased sedimentation rate
d. Painless subcutaneous nodules

5.36 A nurse is preparing child for discharge after diagnosed with rheumatic fever that is the following instruction أسئلة سابقة

a. Only give penicillin for dental work
b. Give aspirin for management chorea
c. Give penicillin until 21 years old
d. Give penicillin for one month total

5.37 Which of the following nursing diagnosis is most appropriate for child with rheumatic fever? أسئلة سابقة

a. High risk for injury
b. Imbalance munition more than requirement
c. Delayed growth and development
d. Impaired gas exchange

5.38 In case of rheumatic heart fever with Myocarditis, the chest x-ray reveals normal heart size أسئلة سابقة

a. True
b. False

5.39 Rheumatic fever is inflammation of the joints, heart, skin, and nervous system that occurs in children as a complication of untreated streptococcal infection of the throat. While most people with rheumatic fever recover, which part of the body may be permanently damaged in a small percentage of people? أسئلة سابقة

a. Digestive tract
b. Ears
c. Heart
d. Lungs

5.40 Which of the following is usually the first symptom of rheumatic fever? أسئلة سابقة

a. Chest pain or palpitations
b. Jerky, uncontrollable movements
c. Joint pain and fever
d. Rash

5.41 Penicillin is safely given to patients by the IV route. (True / False) أسئلة سابقة

a. True
b. False

5.42 Each of the following drugs is used to treat rheumatic fever in children EXCEPT: أسئلة سابقة

a. Acetaminophen
b. Antibiotics
c. Anti-inflammatory drugs
d. Corticosteroids

5.43 Patient with polyarthritis and carditis without evident of CHF (congestive heart failure) is treated with أسئلة سابقة

a. IV salicylates
b. Oral salicylates
c. Corticosteroid
d. Both B and C

5.44 Which symptom presented is NOT a sign of rheumatic fever? أسئلة سابقة

a. Truncal rash
b. Joint tenderness
c. Changes in vision
d. Nausea

5.45 Rheumatic fever is caused by which of the following bacteria? أسئلة سابقة

a. Pseudomonas
b. Staphylococcus
c. Streptococcus
d. Shingella

5.46 Which of the following factors in the patient's history and physical is LEAST relevant to the diagnosis of Rheumatic fever? أسئلة سابقة

a. Sore throat from 3 weeks previous
b. Living in the current conditions of Flint, MI
c. The mother's history of type-2 diabetes
d. A possible genetic factor from the father who's history is unknown

5.47 The provider suspects rheumatic fever and orders blood work. Which results indicate inflammation and cardiac involvement? (Select all that apply) أسئلة سابقة

a. Elevated C-reactive protein
b. Decreased erythrocyte sedimentation rate
c. Elevated WBC count
d. Decreased hemoglobin and hematocrit

5.48 **According to the Jones criteria, which sets of findings would indicate a positive diagnosis of rheumatic fever? (Select all that apply)** أسئلة سابقة

a. Carditis, fever, and an elevated WBC count
b. Positive strep throat culture, arthritis, chorea
c. Recent scarlet fever, carditis, fever, arthralgia
d. Elevated C-reactive protein, carditis, fever

5.49 **Acute Rheumatic Fever (ARF) is primarily caused by which bacterial species?** أسئلة سابقة

a. Staphylococcus aureus
b. Streptococcus pneumoniae
c. Streptococcus agalactiae
d. Streptococcus pyogenes (Group A Streptococcus)

5.50 **Molecular mimicry in Acute Rheumatic Fever refers to:** أسئلة سابقة

a. The ability of bacteria to mimic host cells.
b. The interaction between antibodies and antigens.
c. The cross-reactivity between bacterial antigens and host tissues.
d. The production of toxins by Streptococcus pyogenes.

5.51 **Which of the following clinical manifestations is NOT a major criterion for diagnosing Acute Rheumatic Fever?** أسئلة سابقة

a. Carditis
b. Chorea
c. Elevated C-reactive protein
d. Subcutaneous nodules

5.52 **Patient with polyarthritis and carditis with evident cardiomegaly is treated with** أسئلة سابقة

a. IV salicylates
b. Oral salicylates
c. Corticosteroid
d. Adrenaline

5.53 **Which of the following in not considered among supportive therapies for patient with carditis** أسئلة سابقة

a. Digoxin
b. Diuretics
c. Oxygen
d. Increasing fluid and salt intake

5.54 **Which clinical manifestation of Acute Rheumatic Fever involves rapid, jerky movements and behavioral changes?** أسئلة سابقة

a. Carditis
b. Arthritis
c. Chorea
d. Subcutaneous nodules

5.55 **Which genetic marker can influence an individual's susceptibility to developing Acute Rheumatic Fever after a streptococcal infection?** أسئلة سابقة

a. HLA-B27
b. Rh factor
c. ABO blood type
d. Genetic markers associated with the immune response

5.56 Which of the following is a major clinical manifestation of Acute Rheumatic Fever that involves inflammation of the heart? أسئلة سابقة

a. Chorea
b. Arthritis
c. Carditis
d. Subcutaneous nodules

5.57 Which of the following is NOT a potential complication of Acute Rheumatic Fever? أسئلة سابقة

a. Rheumatic heart disease
b. Glomerulonephritis
c. Meningitis
d. Chorea

5.58 Which of the following is a minor criterion for diagnosing Acute Rheumatic Fever? أسئلة سابقة

a. Carditis
b. Arthritis
c. Elevated erythrocyte count
d. Subcutaneous nodules

5.59 What is the gold standard for diagnosing Group A Streptococcus infection in the laboratory? أسئلة سابقة

a. Rapid strep test
b. Gram staining
c. Throat culture
d. Molecular PCR test

5.60 Which of the following medications is NOT commonly used in the treatment of Acute Rheumatic Fever? أسئلة سابقة

a. Penicillin
b. Nonsteroidal anti-inflammatory drugs (NSAIDs)
c. Corticosteroids
d. Antibiotics targeting Gram-negative bacteria

5.61 Rheumatic fever primarily affects which of the following body systems? أسئلة سابقة

a. Digestive system
b. Musculoskeletal system
c. Respiratory system
d. Nervous system

5.62 Which of the following is the characteristic feature of rheumatic fever? أسئلة سابقة

a. Joint pain and swelling
b. Skin rash
c. Heart murmurs
d. All of the above

5.63 Which of the following age groups are at higher risk for rheumatic fever? Select the correct answer أسئلة سابقة

a. Adults 50 and above
b. Children ages 5-15
c. Infants
d. Women ages 20-40

5.64 The best way to prevent rheumatic fever is: أسئلة سابقة

a. Avoiding contact with infected individuals
b. Getting vaccinated
c. Early diagnosis and treatment of streptococcal infections
d. Washing hands frequently

5.65 Rheumatic fever can lead to long-term complications, primarily affecting the..... أسئلة سابقة

a. Heart
b. Kidneys
c. Liver
d. Lungs

5.66 The condition characterized by scarring and damage to the heart valves as a consequence of rheumatic fever is called..... أسئلة سابقة

a. Coronary artery disease
b. Rheumatic heart disease
c. Rheumatoid arthritis
d. Septicemia

5.67 Which of the following nursing diagnoses is most appropriate for a child with rheumatic fever? ميدترم 2022 (أطفال نظري)

a. High risk for injury
b. Imbalanced nutrition: more than requirements
c. Delayed growth and development
d. Impaired gas exchange

5.68 A nurse is preparing a child for discharge after being diagnosed with rheumatic fever. Which is the correct instruction? ميدترم 2022 (أطفال نظري)

a. Only give penicillin for dental work
b. Give aspirin for management of chorea
c. Give penicillin until 21 years old
d. Give penicillin for one month total

5.69 Which of the following instructions would the nurse include in a teaching plan that focuses on prevention of rheumatic fever? ميدترم 2024 (ترم أول 2023-2024) (أطفال نظري)

a. Use corticosteroids to reduce inflammation
b. Provide antibiotic before dental work
c. Give penicillin to patients for treatment
d. Seek immediate treatment for throat infection

5.70 One minor manifestation of rheumatic fever is: ميدترم 2024 (ترم أول 2023-2024) (أطفال نظري)

a. Erythema marginatum
b. Polyarthritis and chorea
c. Increased sedimentation rate
d. Painless subcutaneous nodules

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

5.N1 Rheumatic fever is an autoimmune reaction that follows infection with: NCLEX

a. Staphylococcus aureus
b. Group A beta-hemolytic streptococcus
c. Pneumococcus
d. E. coli

5.N2 Select ALL MAJOR Jones criteria. SELECT ALL

a. Carditis
b. Polyarthritis
c. Chorea
d. Erythema marginatum
e. Subcutaneous nodules
f. Elevated ESR

5.N3 **Secondary prophylaxis after rheumatic fever usually consists of:** NCLEX

a. Oral aspirin for one month
b. Benzathine penicillin IM every 3–4 weeks for years
c. Corticosteroids for life
d. Antibiotics only during dental work

5.N4 **A child with RF has chorea. The priority nursing diagnosis is:** NCLEX

a. Impaired gas exchange
b. Risk for injury
c. Constipation
d. Excess fluid volume

5.N5 **Which finding indicates carditis in a child with rheumatic fever?** NCLEX

a. New heart murmur and tachycardia out of proportion to fever
b. Migratory joint pain
c. Ring-shaped rash
d. Painless nodules on extensor surfaces

5.N6 **The joint involvement in rheumatic fever is typically:** NCLEX

a. Small joints, symmetric and deforming
b. Migratory polyarthritis of large joints, non-deforming
c. Spinal only
d. Single joint with permanent damage

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Cause	Autoimmune reaction 2–3 weeks after group A beta-hemolytic strep pharyngitis.	**Molecular mimicry**	Cross-reactivity between streptococcal antigens and host tissues.
Major Jones criteria	Carditis, polyarthritis, chorea, erythema marginatum, subcutaneous nodules.	**Minor criteria**	Fever, arthralgia, raised ESR/CRP, prolonged PR interval.
Evidence of strep	Positive throat culture (gold standard) or raised ASO titer.	**Arthritis**	Migratory, large joints, non-deforming.
Chorea (Sydenham)	Involuntary jerky movements, emotional lability; risk for injury.	**Carditis**	Only manifestation causing permanent damage → rheumatic heart disease (mitral).
Treatment	Penicillin, salicylates for arthritis, steroids for carditis with cardiomegaly/CHF, bed rest.	**Secondary prophylaxis**	Benzathine penicillin IM every 3–4 weeks (often until 21 years).
Primary prevention	Prompt treatment of strep throat.	**Age group**	Children 5–15 years.

Answer Key — All Lectures

الإجابات

1.1. b — Acting in the best interest of the child
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

1.2. c — If they are legally emancipated
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

1.3. b — Report it to the appropriate authorities immediately
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

1.4. b — If the child's condition is life-threatening
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

1.5. b — Making the decision that benefits the child the most
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

1.6. b — Sexually transmitted infections (STIs)
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

1.7. b — Power of attorney
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

1.8. c — Any parent with legal custody can consent
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

1.9. c — The court intervenes in significant cases
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

1.10. c — They may not have the full ability to make complex decisions
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

1.11. b — Respect the adolescent's privacy unless there is a safety concern
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

1.N1. b — Justice
Justice = fair distribution of care based on need and priority.

1.N2. b — Politely refuse the gift
Professional responsibilities include refusing gifts from patients or families.

1.N3. a, b, c, d — Right to live; Right to privacy; Protection from danger; Right to treatment
The six rights: live, privacy, protection from danger, treatment, feeding and interacting with others.

1.N4. b — Get down to the child's eye level and build trust with the parent present
Putting yourself at the child's level and involving parents reduces fear and builds trust.

1.N5. b — Seek a court order/intervention while documenting the refusal
The child's best interest overrides parental autonomy in life-threatening situations; legal intervention is sought.

1.N6. b — Refused, with counselling and reporting if harm was done
FGM is illegal (Law 126/2008, strengthened 2016).

1.N7. a, b, d — Life-threatening emergency; Abandoned (street) child with pneumonia; Child protection case of abuse
Emergencies, abandoned children and protection cases allow treatment without parental consent; elective surgery does not.

1.N8. b — Notify the Ministry of Health and enforce isolation
Public Health Law mandates isolation of infectious TB until treated.

2.1. d — Give dose if heart rate is 100 beats/min

2.2. a — Blood shunt from left to right

2.3. a — True

2.4. a — Assess heart rate

2.5. d — Susceptibility to respiratory infection

2.6. a — Susceptibility to respiratory infection

2.7. c — Bradycardia and nausea

2.8. d — Apical pulse

2.9. c — Tetralogy of Fallot (TOF)

2.10. a — Dyspnea, cyanosis, and poor feeding
الكويز ده مفيهوش إجابة معلّمة — إجابة مقترحة.

2.11. a — Ventricular septal defect

2.12. d — Apical pulse

2.13. d — Tetralogy of Fallot (TOF)

2.14. a — Place the child in a knee-to-chest position

2.15. a — Blalock-Taussig shunt

2.16. b — Persistent ductus arteriosus (PDA)

2.17. b — To reduce pulmonary vascular disease risk

2.18. c — Hypoplastic left heart syndrome (HLHS)

2.19. c — 3-5 years

2.20. b — Transposition of the great arteries (TGA)

2.21. c — Circulatory collapse after ductal closure

2.22. c — Prostaglandin E1

2.23. b — Ejection systolic murmur with a click

2.24. c — Surgical correction

2.25. b — Tetralogy of Fallot (TOF)

2.26. b — Cyanotic heart disease

2.27. c — Atrial septal defect

2.28. b — Ventricular septal defect (VSD)

2.29. c — Cyanosis at birth

2.30. b — Echocardiography

2.31. b — Small left-sided structures

2.32. b — Maintaining ductal patency

2.33. b — Administer prostaglandin E1

2.34. a — Right ventricular outflow tract obstruction

2.35. a — Surgical repair
الملف معلّم Surgical repair، لكن العلاج المفضّل للـ PS مع gradient > 64 mmHg هو balloon dilatation (valvuloplasty). الجراحة للحالات اللي البالون فشل فيها.

2.36. b — Atrioventricular septal defect (AVSD)

2.37. d — Apical pulse.

2.38. a — True

2.39. a — Ventricular septal defect

2.40. a — Assess heart rate.

2.41. a — 100 beat/min
نفس السؤال ظهر بـ 0 من (pediatric questions ملف) في صورة امتحان 1 مع اختيار 100 — المراجع بتقول امنع الجرعة لو النبض > 90–110 في الرضيع، فاعتمد 100 زي بنك الدفعة.

2.42. a — Susceptibility to respiratory infection

2.43. c — Bradycardia and nausea

2.44. b — Record the heart rate and administer the medication

2.45. c — Increased respiratory rate

2.46. a — Ture

2.47. c — TGA

2.48. a — Increased urinary output

2.49. b — Weight gain

2.50. a — True

2.51. d — All of Above

2.52. d — Congestive heart failure

2.53. a — Patent dictus arteriosus

2.54. a — Increased heart size

2.55. a — Prostaglandin E1

2.56. a — Anoxic spells

2.57. c — Prostaglandin E1

2.58. a — True

2.59. d — Dopamine

2.60. e — CHF

2.61. d — Younger maternal age

2.62. b — False

2.63. a — True

2.64. c — Tetralogy of Fallot

2.65. b — False

2.66. e — A and C

2.67. a — Abnormal opening between the Rt. And Lt.Ventricles.

2.68. c — Bradypnea

2.69. b — False

2.70. b — False

2.71. a — True

2.72. c — Prostaglandin E1

2.73. b — False

2.74. d — Aorta originating from the right ventricle, and pulmonary originating from the left ventricle.

2.75. c — Left thoracicotomy

2.76. b — False

2.77. c — Atrial septal defect (ASD)

2.78. d — Cyanotic CHD

2.79. c — Anoxic spells

2.80. e — Place infant in prone position

2.81. d — All of the above

2.82. a — Cerebral thrombosis

2.83. c — Increase pulmonary artery blood flow.

2.84. d — CHF

2.85. b — Decreased systemic venous circulation

2.86. d — All of the above

2.87. d — All of the above

2.88. d — Pulmonary Venous Congestion

2.89. c — Systemic Venous Congestion

2.90. d — Decreased urine output (oliguria).

2.91. d — Propranolol
Beta-blockers (propranolol) مش من علاج الـ CHF الحاد في الأطفال حسب البنك.

2.92. a — Replacing regular nipples with easy-to-suck ones

2.93. c — Increase heart rate

2.94. d — Tachycardia

2.95. e — All of the above.

2.96. a — True

2.97. c — Tetralogy of Fallot (TOF)
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.98. c — Ventricular septal defect (VSD)
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.99. b — Decreased systemic oxygenation
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.100. c — Echocardiography (ECHO)
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.101. c — Diabetes
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.102. b — The aorta and pulmonary artery are switched
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.103. c — Weak femoral pulses and hypertension in upper limbs
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.104. c — Prostaglandin E1
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.105. b — Chronic hypoxia leading to increased erythropoiesis
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.106. c — Avoiding teratogenic medications
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2.N1. c — Ventricular septal defect
VSD, ASD and PDA shunt blood left to right — acyanotic, with increased pulmonary flow.

2.N2. b — Pulmonary stenosis, VSD, overriding aorta, right ventricular hypertrophy
TOF = PS + VSD + overriding aorta + RVH.

2.N3. a, b, c, d — Knee-chest position; Calm, comforting approach; 100% oxygen; Morphine as prescribed
Knee-chest increases systemic resistance; oxygen and morphine reduce hypoxia and infundibular spasm.

2.N4. c — 1 full minute
Count apical pulse for one full minute; hold if below the ordered limit (infant ~90–110).

2.N5. c — Prostaglandin E1
PGE1 maintains ductal patency; indomethacin/ibuprofen close it.

2.N6. b — Small frequent feedings with soft easy-to-suck nipples
Small frequent feeds with soft nipples reduce energy expenditure; limit feeding time to about 30 minutes.

3.1. c — Inability to jump or stand on one foot
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.2. b — Use of accessory muscles for breathing
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.3. b — Blood culture, urine culture, LP and admit for IV antibiotics
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.4. b — Compassionate tone, simple terms and offering next steps
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.5. d — Spinal muscular atrophy
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.6. c — Principle of integration
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.7. a — Statistical information, client profile, health history, family history, review of systems, lifestyle and life patterns
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.8. b — Cultural barriers
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.9. b — Increased intracranial pressure (ICP)
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3.N1. b — Inspection, auscultation, percussion, palpation
Auscultation precedes palpation/percussion in the abdomen so bowel sounds are not altered.

3.N2. b — Subjective data
Subjective data = what the patient or mother says; objective = what the examiner finds.

3.N3. a, b, c, d — Upslanting palpebral fissures; Flattened nasal bridge; Brushfield spots; Epicanthal fold
Frontal bossing/maxillary expansion is chipmunk facies of thalassemia.

3.N4. b — Increased intracranial pressure
A tense bulging fontanel indicates raised ICP (e.g., hydrocephalus, meningitis); sunken fontanel suggests dehydration.

3.N5. b — Warm the hands, position supine and talk to relax the child
Warm hands, supine position and conversation relax abdominal muscles.

3.N6. b — Chronic hemolytic anemia (thalassemia)
Extramedullary hematopoiesis causes frontal bossing and maxillary expansion.

4.1. c — Impaired swallowing related to inflammation

4.2. d — Vigilant assessment, racemic epinephrine, and corticosteroids

4.3. b — Observed for signs of hemorrhage

4.4. c — Mobilizing/clearing the child's secretions
نص الاختيار الصح كان مقصوص في صورة الميدترم («...the child's secretions») — اتكتب بالمعنى.

4.5. b — Nasal congestion

4.6. a — Streptococcus pneumoniae

4.7. b — Supportive care

4.8. b — Wheezing

4.9. b — Passive smoking

4.10. a — Chest X-ray

4.11. b — Salbutamol

4.12. b — Croup

4.13. b — Influenza vaccine

4.14. c — Wheezing and respiratory distress

4.15. c — Bronchitis

4.16. b — Polluted air

4.17. b — Nasal flaring

4.18. a — Antibiotics

4.19. a — RSV (Respiratory Syncytial Virus)

4.20. b — Pleural effusion

4.21. b — Relax airway muscles

4.22. b — Cyanosis

4.23. b — Barking cough

4.24. b — Supportive care with humidified air

4.25. a — Sudden onset of coughing or choking

4.26. c — Bronchoscopy

4.27. a — Silent chest

4.28. d — School-aged children

4.29. c — Crackles

4.30. b — Malnutrition

4.31. b — RSV
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.32. d — Stridor and barking cough
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.33. b — Foreign body aspiration
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.34. b — Streptococcus pneumoniae
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.35. a — Inhaled SABA
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.36. c — Group A Streptococcus (S. pyogenes)
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.37. b — Chest X-ray
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.38. c — Grunting respiration
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.39. c — Mechanical upper-airway obstruction during sleep
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.40. c — Reversible airway obstruction
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4.41. b — Chest physiotherapy
إجابة الطالب في الصورة (chest physiotherapy) — إجابة مقترحة.

4.N1. b — Croup (laryngotracheobronchitis)
Croup: barking cough, stridor, hoarseness; treat with humidified air, steroids, racemic epinephrine.

4.N2. b — RSV
RSV; management is supportive (O2, hydration, suction) — antibiotics not indicated.

4.N3. a, b, c, d — Nasal flaring; Intercostal retractions; Grunting; Tachypnea
Cyanosis and decreasing consciousness indicate failure.

4.N4. b — Severe life-threatening exacerbation
Minimal air movement — status asthmaticus.

4.N5. a — Frequent swallowing (bleeding)
Frequent swallowing signals hemorrhage; avoid red fluids and straws.

4.N6. a — Inhaled salbutamol
Short-acting beta-2 agonist relaxes airway smooth muscle.

5.1. b — An inflammatory disease following a bacterial infection

5.2. a — Streptococcus pyogenes

5.3. b — Fever, joint pain, and skin rash

5.4. a — Subcutaneous nodules

5.5. a — Throat swab culture

5.6. b — Preventing recurrence and complications

5.7. a — Valvular damage

5.8. b — Early treatment of streptococcal infections

5.9. a — Penicillin

5.10. b — Children aged 5-15 years

5.11. b — Cardiovascular system

5.12. c — Skin ulcers

5.13. a — Echocardiography

5.14. a — Fever

5.15. a — Uncontrollable jerky movements

5.16. a — Autoimmune cross-reactivity

5.17. b — Penicillin

5.18. a — Heart murmur

5.19. d — Until age 21
الملف معلّم — 5 carditis: بدون سنين أو لحد سن 21 (الأطول منهم) until age 21.

5.20. a — True

5.21. b — False

5.22. b — False

5.23. b — False

5.24. a — True

5.25. c — 2–3 weeks
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

5.26. b — Knees, ankles, elbows and wrists
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

5.27. b — Pain that moves from joint to joint (migratory)
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

5.28. d — Headache
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

5.29. b — Hot or tender joints
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

5.30. b — Musculoskeletal, cardiovascular and nervous
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

5.31. b — Fever and joint pain
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

5.32. b — Erythema marginatum
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

5.33. c — Penicillin
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

5.34. d — Nausea
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

5.35. c — Increased sedimentation rate

5.36. c — Give penicillin until 21 years old

5.37. a — High risk for injury

5.38. b — False

5.39. c — Heart

5.40. c — Joint pain and fever

5.41. b — False

5.42. a — Acetaminophen

5.43. b — Oral salicylates

5.44. c — Changes in vision

5.45. c — Streptococcus

5.46. c — The mother's history of type-2 diabetes

5.47. a, c — Elevated C-reactive protein; Elevated WBC count

5.48. b, c — Positive strep throat culture, arthritis, chorea; Recent scarlet fever, carditis, fever, arthralgia

5.49. d — Streptococcus pyogenes (Group A Streptococcus)

5.50. c — The cross-reactivity between bacterial antigens and host tissues.

5.51. c — Elevated C-reactive protein

5.52. c — Corticosteroid

5.53. d — Increasing fluid and salt intake

5.54. c — Chorea

5.55. d — Genetic markers associated with the immune response

5.56. c — Carditis

5.57. c — Meningitis

5.58. b — Arthritis

5.59. c — Throat culture

5.60. d — Antibiotics targeting Gram-negative bacteria

5.61. b — Musculoskeletal system
إجابة البنك: musculoskeletal (المفاصل أشيع عرض). لكن أخطر وأدوم تأثير على القلب.

5.62. d — All of the above

5.63. b — Children ages 5-15

5.64. c — Early diagnosis and treatment of streptococcal infections

5.65. a — Heart

5.66. b — Rheumatic heart disease

5.67. a — High risk for injury

5.68. c — Give penicillin until 21 years old

5.69. d — Seek immediate treatment for throat infection

5.70. c — Increased sedimentation rate

5.N1. b — Group A beta-hemolytic streptococcus
Molecular mimicry between streptococcal antigens and host tissues (heart, joints, brain, skin) causes RF.

5.N2. a, b, c, d, e — Carditis; Polyarthritis; Chorea; Erythema marginatum; Subcutaneous nodules
The 5 major criteria are carditis, polyarthritis, chorea, erythema marginatum and subcutaneous nodules; elevated ESR/CRP is minor.

5.N3. b — Benzathine penicillin IM every 3–4 weeks for years
Long-acting penicillin prevents recurrence; duration depends on carditis (often until age 21 or longer).

5.N4. b — Risk for injury
Involuntary jerky movements put the child at risk of falls and injury — provide a safe, quiet environment.

5.N5. a — New heart murmur and tachycardia out of proportion to fever
Carditis presents with new murmurs, tachycardia, cardiomegaly or pericardial rub; it is the only manifestation that can cause permanent damage.

5.N6. b — Migratory polyarthritis of large joints, non-deforming
RF arthritis migrates between large joints (knees, ankles, elbows, wrists) and heals without deformity.

PEDIATRIC MEDICINE · NEPHROLOGY · LECTURE 1 · QUESTION BANK

Nephrology

محاضرة أمراض الكلى عند الأطفال — بنك الأسئلة

10	73	58
Past exam questions	NCLEX-style	Flashcards

المصادر اللي اتجمعت منها أسئلة المحاضرة دي:

أسئلة سابقة

جنب كل سؤال مصدره. لو السؤال اتكرر في كذا امتحان مكتوب كل مرة بمصدرها، وعليه علامة "تكرر". الإجابات كلها في آخر الملف.

اللَّهُمَّ لا سَهْلَ إِلَّا ما جَعَلْتَهُ سَهْلًا، وأنتَ تَجْعَلُ الحَزْنَ إذا شِئْتَ سَهْلًا — صلِّ على النبي ﷺ

Part A · Past Exam Questions

أسئلة الدفعات السابقة

1 أسئلة سابقة

Nephrotic syndrome is primarily characterized by:

a. Hematuria
b. High creatinine levels
c. Massive proteinuria
d. Urinary tract obstruction

2 أسئلة سابقة

The most common cause of nephrotic syndrome in children is:

a. FSGS
b. Membranous nephropathy
c. Minimal change disease
d. Lupus nephritis

3 أسئلة سابقة

A key difference between nephritis and nephrosis is:

a. Foamy urine in nephritis
b. Inflammation in nephrosis
c. Edema in nephritis only
d. Inflammation is a main feature of nephritis

4 أسئلة سابقة

Which of the following is a complication of nephrotic syndrome?

a. Hypotension
b. Weak immune system (infection)
c. Hyperkalemia
d. Anemia

5 أسئلة سابقة

Which diagnostic tool confirms kidney tissue damage?

a. CT scan
b. Urinalysis
c. Blood test
d. Kidney biopsy

6 أسئلة سابقة

Nephritis is commonly caused by:

a. Hyperlipidemia
b. Autoimmune diseases
c. Minimal change disease
d. Nephrotoxic drugs only

7 Which medication is used to manage high cholesterol in nephrosis? أسئلة سابقة

a. Furosemide
b. Atorvastatin
c. Penicillin
d. Prednisone

8 In nephritis, which symptom is common? أسئلة سابقة

a. Massive proteinuria
b. Foamy urine
c. Hypertension
d. Severe hyperlipidemia

9 Which dietary recommendation is common for both nephritis and nephrosis? أسئلة سابقة

a. High-sodium intake
b. High-protein diet
c. Low-protein, low-sodium diet
d. No dietary restrictions

10 What is the purpose of corticosteroids in nephritis treatment? أسئلة سابقة

a. Lower blood pressure
b. Control fluid retention
c. Treat infections
d. Reduce inflammation

Part B · NCLEX-Style Practice

أسئلة على طريقة NCLEX (إضافة)

N1 Which function is NOT performed by the kidney? NCLEX

a. Activation of vitamin D (1-alpha hydroxylation)
b. Production of erythropoietin
c. Production of bile
d. Regulation of acid-base balance

N2 The structural and functional unit of the kidney is the: NCLEX

a. Glomerulus only
b. Nephron
c. Renal pelvis
d. Collecting duct

N3 The glomerular filtration barrier consists of (select all): SELECT ALL

a. Fenestrated endothelium
b. Glomerular basement membrane (negatively charged)
c. Podocytes with slit diaphragms
d. Renal capsule
e. Collecting duct epithelium

N4 Normal protein excretion in a child is: NCLEX

a. ≤ 4 mg/m²/hr (P/C ratio < 0.2)
b. > 40 mg/m²/hr
c. P/C ratio > 2
d. 3+ on dipstick

N5 The first-line imaging study for a child with a renal problem is: NCLEX

a. CT abdomen
b. Renal ultrasound
c. MRI
d. Renal biopsy

N6 **Hematuria of glomerular origin is suggested by (select all):** SELECT ALL

a. Cola / smoky urine
b. RBC casts
c. Dysmorphic RBCs
d. Clots in urine
e. Associated proteinuria

N7 **Renal biopsy is examined by (select all):** SELECT ALL

a. Light microscopy
b. Immunofluorescence
c. Electron microscopy
d. Culture
e. Gram stain

N8 **Which hormone produced by the kidney stimulates red blood cell production?** NCLEX

a. Renin
b. Erythropoietin
c. Aldosterone
d. Calcitriol

N9 **A child's urinalysis shows SG 1.010–1.025, pH 6, no protein, 2 RBCs/HPF. The nurse interprets this as:** NCLEX

a. Normal urinalysis
b. Glomerular hematuria
c. Nephrotic-range proteinuria
d. Urinary tract infection

N10 **Which test is used to detect vesicoureteral reflux?** NCLEX

a. Renal ultrasound
b. Voiding cystourethrogram (VCUG)
c. Plain X-ray KUB
d. Urine culture

N11 **Before a percutaneous renal biopsy, the nurse should check (select all):** SELECT ALL

a. Coagulation profile and platelets
b. Blood pressure
c. Signed consent
d. Serum lipase
e. Hemoglobin / blood grouping

N12 **After a renal biopsy, which finding requires immediate reporting?** NCLEX

a. Mild pink urine in the first void
b. Falling blood pressure with rising pulse
c. Mild discomfort at the site
d. Child asking for food

N13 **Bright red urine with clots in a child suggests bleeding from:** NCLEX

a. The glomerulus
b. The lower urinary tract
c. The tubules
d. The liver

N14 **Red urine with a negative dipstick for blood and no RBCs may be caused by (select all):** SELECT ALL

a. Beetroot ingestion
b. Rifampicin
c. Urate crystals in infants
d. Post-streptococcal GN
e. Food dyes

N15 **A 4-year-old with minimal change nephrotic syndrome has generalized edema. Which laboratory finding confirms the diagnosis?** NCLEX

a. Serum albumin 1.8 g/dL with heavy proteinuria
b. Low C3 complement level
c. RBC casts in urine
d. Serum potassium 6.2 mEq/L

N16 **Which findings are part of the definition of nephrotic syndrome? Select all that apply.** SELECT ALL

a. Hypoalbuminemia < 2.5 g/dL
b. Cholesterol > 200 mg/dL
c. Generalized pitting edema
d. Gross hematuria
e. Heavy (nephrotic-range) proteinuria

N17 **The physician orders prednisone 2 mg/kg/day for a child with nephrotic syndrome. The best indicator that the drug is effective is:** NCLEX

a. Increased appetite
b. Urine protein becoming negative / trace for 3 days
c. Rising blood pressure
d. Weight gain

N18 **A child with nephrotic syndrome suddenly develops fever and abdominal pain with tenderness. The nurse suspects:** NCLEX

a. Hypovolemic shock
b. Spontaneous bacterial peritonitis
c. Renal vein thrombosis
d. Steroid psychosis

N19 **Nursing priorities for an edematous child with nephrotic syndrome include (select all):** SELECT ALL

a. Daily weight at the same time
b. Measure abdominal girth
c. Turn frequently and protect edematous skin
d. High-salt diet to improve appetite
e. Keep away from children with infections

N20 **Which complication of nephrotic syndrome results from urinary loss of antithrombin III?** NCLEX

a. Infection
b. Thrombosis (DVT, renal vein)
c. Hypocalcemia
d. Hypertension

N21 **Renal biopsy before steroid treatment in nephrotic syndrome is indicated when (select all):** SELECT ALL

a. Onset below 1 year or above 10 years
b. Persistent hypertension
c. Low C3
d. Gross hematuria
e. Selective proteinuria in a 4-year-old

N22 A child on long-term steroids for nephrotic syndrome is going home. Which instruction is most important? NCLEX

a. Stop prednisone immediately if face becomes puffy
b. Test urine for protein daily and avoid live vaccines
c. Increase salt intake
d. Restrict all protein

N23 Congenital nephrotic syndrome is defined as nephrotic syndrome that presents: NCLEX

a. In the first 3 months of life
b. Between 1 and 10 years
c. After puberty
d. Only after a strep infection

N24 Steroid-resistant nephrotic syndrome is failure to achieve remission after: NCLEX

a. 1 week of prednisone
b. 4–6 weeks of daily prednisone
c. 6 months of therapy
d. One dose of methylprednisolone

N25 Frequent relapses in nephrotic syndrome means: NCLEX

a. ≥ 2 relapses within 6 months or ≥ 4 in 12 months
b. 1 relapse per year
c. Relapse during the first week of treatment
d. Persistent hematuria

N26 The edema of nephrotic syndrome is mainly due to: NCLEX

a. Increased capillary hydrostatic pressure
b. Decreased plasma oncotic pressure from hypoalbuminemia
c. Lymphatic obstruction
d. Heart failure

N27 Where is edema of nephrotic syndrome first noticed in a child? NCLEX

a. Feet in the evening only
b. Periorbital in the morning
c. Hands
d. Sacrum

N28 Hyperlipidemia in nephrotic syndrome occurs because: NCLEX

a. Fat intake is high
b. The liver increases lipoprotein synthesis in response to low albumin
c. Steroids are given
d. The kidney produces cholesterol

N29 Side effects of long-term corticosteroid therapy that the nurse should monitor (select all): SELECT ALL

a. Hypertension
b. Hyperglycemia
c. Cushingoid face and weight gain
d. Growth retardation
e. Hypoglycemia

N30 A child with severe edema and low albumin is ordered IV albumin 20% followed by furosemide. The nurse's priority during infusion is: NCLEX

a. Check for hypokalemia only
b. Monitor for fluid overload: BP, respiration, crackles
c. Encourage oral fluids
d. Restrict all activity

N31 Which immunization is especially recommended for children with nephrotic syndrome when off high-dose steroids? NCLEX

a. Pneumococcal vaccine
b. BCG only
c. Rabies vaccine
d. Yellow fever vaccine

N32 Which nursing diagnosis has the highest priority for a child with anasarca due to nephrotic syndrome? NCLEX

a. Excess fluid volume
b. Knowledge deficit
c. Disturbed body image
d. Activity intolerance

N33 Which history is most expected in a child admitted with acute post-streptococcal glomerulonephritis? NCLEX

a. Sore throat 1–2 weeks ago
b. Measles 6 months ago
c. Recent dog bite
d. Allergy to milk

N34 Classic features of APSGN include (select all): SELECT ALL

a. Cola / tea-colored urine
b. Periorbital edema
c. Hypertension
d. Massive selective proteinuria
e. Oliguria

N35 A child with AGN complains of headache and blurred vision; BP 160/110. The nurse's priority is: NCLEX

a. Encourage fluids
b. Notify the physician — suspect hypertensive encephalopathy
c. Give a high-protein snack
d. Ambulate the child

N36 Which laboratory result supports the diagnosis of APSGN? NCLEX

a. Low C3 with raised ASO titre
b. Serum albumin 1.5 g/dL
c. Cholesterol 450 mg/dL
d. Negative urine for RBCs

N37 Fluid allowance for an oliguric child with AGN is calculated as: NCLEX

a. Maintenance fluids plus 20%
b. Previous day's urine output + insensible loss (≈400 mL/m^2/day)
c. Unlimited oral fluids
d. Nothing by mouth for 48 hours

N38 **The earliest sign of improvement in a child with AGN is:** NCLEX

a. Disappearance of microscopic hematuria
b. Increased urine output (diuresis)
c. Normal C3
d. Weight gain

N39 **Nursing care of a child with AGN includes (select all):** SELECT ALL

a. Daily weight and strict I&O
b. BP monitoring every 4 hours
c. Low-salt diet
d. Bed rest during the acute oliguric phase
e. Encourage high-potassium foods

N40 **Indications for renal biopsy in suspected APSGN include:** NCLEX

a. C3 still low after 8 weeks or normal at onset
b. Tea-colored urine in the first week
c. Periorbital edema
d. Positive throat culture

N41 **The pathogenesis of APSGN is best described as:** NCLEX

a. Direct bacterial invasion of the kidney
b. Immune complex deposition in the glomeruli
c. Obstruction of the ureter
d. Toxic drug injury

N42 **Hypertension in acute glomerulonephritis is mainly due to:** NCLEX

a. Renin excess only
b. Salt and water retention from decreased GFR
c. Anxiety
d. Steroid therapy

N43 **Which complications of APSGN require ICU admission? (select all)** SELECT ALL

a. Hypertensive encephalopathy
b. Acute heart failure
c. Acute kidney injury with hyperkalemia
d. Mild periorbital edema
e. Microscopic hematuria

N44 **Antibiotic therapy in APSGN is given to:** NCLEX

a. Cure the kidney inflammation
b. Eradicate remaining streptococci and prevent spread
c. Lower blood pressure
d. Reduce proteinuria

N45 Which statement about the prognosis of APSGN in children is correct? NCLEX

a. Most children (> 95%) recover completely
b. Most progress to chronic kidney disease
c. Relapses are common
d. Steroids are needed for recovery

N46 A child with AGN is oliguric with K^+ 6.5 mEq/L. Which food should be avoided? NCLEX

a. Rice
b. Bananas and oranges
c. Apples
d. White bread

N47 Compared with nephrotic syndrome, acute nephritis has (select all): SELECT ALL

a. Gross hematuria
b. Hypertension
c. Low C3
d. Massive proteinuria
e. Marked hyperlipidemia

N48 The best way to evaluate fluid status in a child with AGN is: NCLEX

a. Daily weight on the same scale
b. Skin color
c. Thirst
d. Number of meals

N49 The most common organism causing UTI in children is: NCLEX

a. Klebsiella
b. Escherichia coli
c. Proteus
d. Pseudomonas

N50 Which presentation of UTI is typical in a neonate? NCLEX

a. Dysuria and frequency
b. Fever or temperature instability, poor feeding, jaundice
c. Flank pain
d. Enuresis

N51 Predisposing factors for UTI in children include (select all): SELECT ALL

a. Vesicoureteral reflux
b. Posterior urethral valves
c. Uncircumcised male infant
d. Constipation
e. Drinking plenty of water

N52 A urine culture from a suprapubic aspirate is diagnostic of UTI when it shows: NCLEX

a. > 100,000 CFU only
b. Any bacterial growth
c. Mixed growth of 3 organisms
d. No growth

N53 Which instruction helps prevent recurrent UTI in a school-age girl? NCLEX

a. Wipe from back to front
b. Void regularly and completely; drink enough fluids
c. Take bubble baths
d. Hold urine as long as possible

N54 **Signs pointing to upper UTI (pyelonephritis) rather than cystitis are (select all):** SELECT ALL

a. High fever with rigors
b. Loin pain / tenderness
c. Low-grade or no fever
d. Vomiting and toxic look
e. Suprapubic pain only

N55 **Which urine collection method has the highest contamination rate?** NCLEX

a. Suprapubic aspiration
b. Urinary catheter
c. Urine bag
d. Mid-stream clean catch

N56 **Vesicoureteral reflux increases the risk of:** NCLEX

a. Recurrent pyelonephritis and renal scarring
b. Nephrotic syndrome
c. Hypospadias
d. Wilms tumor

N57 **Indications for IV antibiotics in a child with UTI include (select all):** SELECT ALL

a. Age below 3 months
b. Toxic appearance or vomiting
c. Suspected pyelonephritis with dehydration
d. Mild dysuria in a 10-year-old
e. Urosepsis

N58 **The nurse teaches parents to collect a clean-catch urine specimen. Which instruction is correct?** NCLEX

a. Collect the first drops of urine
b. Clean the genital area, discard the first part, then collect midstream
c. Keep the sample at room temperature for 6 hours
d. Use any clean jar at home

N59 **Which finding on urinalysis supports UTI?** NCLEX

a. Positive leukocyte esterase and nitrites
b. Heavy proteinuria
c. RBC casts
d. High specific gravity only

N60 **Prophylactic antibiotics for recurrent UTI are usually:** NCLEX

a. Low-dose nitrofurantoin or TMP-SMX at night
b. IV ceftriaxone daily
c. High-dose amoxicillin three times daily
d. Gentamicin IM weekly

N61 **A child with severe dehydration from gastroenteritis becomes oliguric. This type of AKI is:** NCLEX

a. Pre-renal
b. Renal (intrinsic)
c. Post-renal
d. Chronic

N62 A child with AKI has serum K^+ 7.2 mEq/L with ECG changes. The first drug to give is: NCLEX

a. Furosemide
b. Calcium gluconate 10% slowly IV
c. Sodium bicarbonate orally
d. Kayexalate rectally

N63 Indications for dialysis in AKI include (select all): SELECT ALL

a. Fluid overload with pulmonary edema
b. Refractory hyperkalemia
c. Uremic encephalopathy
d. Severe metabolic acidosis not responding
e. Mild rise in urea only

N64 Which nursing measure is most important for a child in the oliguric phase of AKI? NCLEX

a. Encourage oral fluids freely
b. Strict intake and output with daily weight
c. High-potassium diet
d. Ambulate frequently

N65 Laboratory findings expected in AKI include (select all): SELECT ALL

a. Raised urea and creatinine
b. Metabolic acidosis
c. Hyperkalemia
d. Hypophosphatemia
e. Hypocalcemia

N66 A child with posterior urethral valves develops anuria. This AKI is: NCLEX

a. Pre-renal
b. Renal
c. Post-renal (obstructive)
d. Hepatorenal

N67 Oliguria in a child is defined as urine output less than: NCLEX

a. 0.5–1 mL/kg/hr
b. 3 mL/kg/hr
c. 2 L/day
d. 5 mL/kg/hr

N68 Which drugs are nephrotoxic and can cause intrinsic AKI? (select all) SELECT ALL

a. Aminoglycosides (gentamicin)
b. NSAIDs
c. Contrast media
d. Paracetamol in normal dose
e. Amphotericin B

N69 Hemolytic uremic syndrome causes AKI with: NCLEX

a. Hemolytic anemia, thrombocytopenia and renal failure after bloody diarrhea
b. Massive proteinuria and edema
c. Fever and flank pain
d. Hydronephrosis

N70 ECG change typical of hyperkalemia is: NCLEX

a. Peaked (tall tented) T waves
b. Prominent U waves
c. ST depression only
d. Prolonged QT from hypocalcemia only

N71 During the diuretic (recovery) phase of AKI, the nurse should watch for: NCLEX

a. Hyperkalemia
b. Dehydration and hypokalemia
c. Fluid overload
d. Hypertension from salt

N72 Peritoneal dialysis is often preferred in infants because: NCLEX

a. It is technically simple and needs no vascular access
b. It removes fluid faster than any method
c. It has no infection risk
d. It does not need a catheter

N73 Diet for a child with AKI in the oliguric phase should be (select all): SELECT ALL

a. Adequate calories mainly from carbohydrate and fat
b. Restricted potassium
c. Restricted phosphate
d. High sodium
e. Controlled protein

Part C · Quizlet Flashcards

كروت مراجعة سريعة

غطّي العمود الأبيض بورقة واختبر نفسك: شوف المصطلح وقول التعريف.

Kidney functions	Water & electrolytes, acid-base, waste excretion, BP (renin), erythropoietin, vitamin D activation.	**Nephron**	Structural & functional unit; ~1 million per kidney; glomerulus + tubules.
Cortex vs medulla	Cortex: glomeruli + convoluted tubules. Medulla: loops of Henle + collecting ducts (pyramids).	**Filtration barrier**	Fenestrated endothelium + GBM (type IV collagen, negative charge) + podocytes.
Urinalysis normal	pH 5–7, SG 1015–1025, no protein, RBC < 5/HPF, no casts.	**Blood tests (KFTs)**	Urea & creatinine, electrolytes, albumin, cholesterol, ABG, CBC, C3.
Imaging	US first-line; X-ray for stones; VCUG for reflux; CT/MRI; DMSA for scars.	**Renal biopsy 'LIE'**	Light microscopy, Immunofluorescence, Electron microscopy.
Proteinuria	Abnormal protein in urine; normal ≤ 4 mg/m²/hr; nephrotic > 40 mg/m²/hr or UP/C > 2.	**Hematuria**	> 5 RBCs/HPF; gross (visible) or microscopic; glomerular (cola, casts) vs non-glomerular (red, clots).
Nephrotic syndrome	Heavy proteinuria + hypoalbuminemia (<2.5 g/dL) + hyperlipidemia (>200 mg/dL) + generalized edema (LEAP).	**Nephrotic-range proteinuria**	> 40 mg/m²/hr or UP/C ratio > 2 (normal ≤ 4 mg/m²/hr, P/C < 0.2).
Idiopathic NS	90% of childhood NS; MCNS is ~85% of idiopathic cases.	**MCNS**	Minimal change NS: age 2–6 yrs, selective proteinuria, normal C3, steroid-responsive.

Secondary NS	Due to systemic disease: SLE, diabetes, HSP, infections (HBV, HCV), drugs.
Urine in NS	Dark, foamy/frothy, 3+/4+ protein, selective (albumin).
Complications 'I THE H'	Infection (pneumococcal peritonitis), Thrombosis, Hypovolemia, Electrolyte disturbance, Hyperlipidemia.
Steroid therapy	Prednisone 2 mg/kg/day × 4 weeks then taper; resistant → biopsy + cyclosporine/tacrolimus.
Remission	Urine protein nil/trace for 3 consecutive days.
Relapse	Recurrence of proteinuria ≥ 3+ for 3 days — a common complication.
Diet in NS	Salt restriction, normal protein, low fat; fluid restriction only if severe edema.
Nursing priority	Daily weight, abdominal girth, I&O, skin care of edema, infection prevention.
APSGN	Most common cause of gross hematuria in children; after group A β-hemolytic strep (M types 4, 12, 49).
Latent period	Throat infection → 1–2 weeks; skin (impetigo) → 3–6 weeks.
Peak age	5–12 years; more in boys (≈2:1).
HEAL	Hematuria (cola/tea), Edema (periorbital), Azotemia/oliguria, Low C3 + hypertension.
Heart failure signs '3T'	Tachycardia, Tachypnea, Tender liver.
Hypertensive encephalopathy	Headache, vision changes, vomiting, confusion, convulsions — emergency.
Complications 'HER'	Hypertension/encephalopathy, Electrolyte (hyperkalemia), Renal failure, + CHF.
Urine in AGN	Cola/smoky, RBC casts, mild–moderate proteinuria.
C3 in APSGN	Low, returns to normal within 8 weeks; ASO / anti-DNase B raised.
Fluid rule	Intake = urine output + insensible loss (400 mL/m²/day).
Treatment 'DRIP'	Diet & rest (salt/fluid restriction), Relieve HTN & edema (diuretics, antihypertensives), ICU if needed, Problems/complications.
First sign of recovery	Diuresis (increased urine output); microscopic hematuria may last months.
Nephritis vs nephrosis	Nephritis = inflammation, hematuria, HTN, low C3. Nephrosis = massive proteinuria, hypoalbuminemia, foamy urine.
UTI	Bacterial growth in the normally sterile urinary system.
Lower vs upper UTI	Cystitis: little/no fever. Pyelonephritis: high fever, loin pain, toxic child.
Organisms 'KEEPS'	Klebsiella, E. coli (~80%), Enterococcus, Proteus, Staph & Pseudomonas.
Route	Mostly ascending; hematogenous in neonates.
Predisposing factors	Obstruction (PUV), VUR, hurried micturition, uncircumcised male, constipation.

Neonate/infant signs	Fever or instability, failure to thrive, vomiting/diarrhea, jaundice, irritability.
Cystitis signs	Dysuria, urgency, frequency, enuresis, suprapubic pain.
Clean-catch culture	> 100,000 CFU of one organism = diagnostic.
SPA / catheter culture	Any growth = diagnostic; > 1 organism suggests contamination.
Urine bag	Non-invasive but high contamination — negative result useful, positive needs confirmation.
Treatment	Cystitis: oral 5 days (TMP-SMX, nitrofurantoin, amoxicillin). Pyelonephritis/infant: IV then oral 10–14 days.
Imaging	Renal US; VCUG for VUR; DMSA for scars.
AKI	Sudden fall in renal function (hours–weeks) → can't keep fluid, electrolyte, acid-base balance.
Pre-renal	Low perfusion: dehydration, shock, hemorrhage, heart failure.
Renal (intrinsic)	ATN, AGN/HUS, nephrotoxic drugs.
Post-renal	Obstruction: PUV, stones, tumours.
Labs	↑ urea & creatinine, hyperkalemia, metabolic acidosis, ↑ phosphate, ↓ calcium.
Fluid challenge	If hypovolemic: normal saline bolus, then reassess urine output.
Hyperkalemia 'C BIG K DROP'	Calcium gluconate, Bicarbonate, Insulin + Glucose, Kayexalate, Diuretics, salbutamol.
Calcium gluconate	1 mL/kg 10% slow IV with ECG monitoring — protects the heart.
Diuretic trial	Furosemide if still oliguric after volume correction.
Dialysis 'A-B-C-U-R'	Anuria/fluid overload, Bleeding, Creatinine rapidly rising, Uremic encephalopathy, Refractory K/acidosis.
Nursing	Strict I&O, daily weight, restrict K & fluids, BP monitoring, watch for seizures.

Answer Key

الإجابات

1. c — Massive proteinuria
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

2. c — Minimal change disease
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

3. d — Inflammation is a main feature of nephritis
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

4. b — Weak immune system (infection)
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

5. d — Kidney biopsy
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

6. b — Autoimmune diseases
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

7. b — Atorvastatin
إجابة مقترحة (المصدر مفيهوش إجابة رسمية). الـ statins بتستخدم في الـ hyperlipidemia المستمرة. الـ (atorvastatin) بيعالج المرض نفسه مش الكوليسترول prednisone.

8. c — Hypertension
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

9. c — Low-protein, low-sodium diet
إجابة مقترحة (المصدر مفيهوش إجابة رسمية). في محاضرة الدكتور: بيتقلل البروتين AGN بياخد بروتين طبيعي أو زيادة شوية، والـ NS الـ azotemia. الاختيار c الملح بيتقلل في الاتنين. هو الأقرب من بس لو في الاختيارات.

10. d — Reduce inflammation
إجابة مقترحة (المصدر مفيهوش إجابة رسمية).

N1. c — Production of bile
Kidney: water, electrolytes, acid-base, waste excretion, BP (renin), EPO, vitamin D activation.

N2. b — Nephron
~1 million nephrons per kidney: glomerulus + tubules.

N3. a, b, c — Fenestrated endothelium; Glomerular basement membrane (negatively charged); Podocytes with slit diaphragms
3-layer shield: endothelium (size), GBM (charge), podocytes.

N4. a — ≤ 4 mg/m²/hr (P/C ratio < 0.2)
Nephrotic range: > 40 mg/m²/hr or P/C > 2.

N5. b — Renal ultrasound
US: size, echogenicity, stones, hydronephrosis. VCUG for reflux.

N6. a, b, c, e — Cola / smoky urine; RBC casts; Dysmorphic RBCs; Associated proteinuria
Clots and bright red urine point to non-glomerular (lower tract) bleeding.

N7. a, b, c — Light microscopy; Immunofluorescence; Electron microscopy
'LIE' approach.

N8. b — Erythropoietin
Loss of EPO in chronic kidney disease causes anemia.

N9. a — Normal urinalysis
Normal: SG 1015–1025, pH 5–7, no protein, RBC < 5/HPF.

N10. b — Voiding cystourethrogram (VCUG)
VCUG shows reflux and posterior urethral valves.

N11. a, b, c, e — Coagulation profile and platelets; Blood pressure; Signed consent; Hemoglobin / blood grouping
Main risk is bleeding; afterwards keep supine, monitor vitals and urine for hematuria.

N12. b — Falling blood pressure with rising pulse
Signs of hemorrhage/shock.

N13. b — The lower urinary tract
Glomerular hematuria is cola/smoky with RBC casts and no clots.

N14. a, b, c, e — Beetroot ingestion; Rifampicin; Urate crystals in infants; Food dyes
Pseudo-hematuria: foods, drugs, urates. APSGN gives true hematuria.

N15. a — Serum albumin 1.8 g/dL with heavy proteinuria
NS = heavy proteinuria + hypoalbuminemia (<2.5 g/dL) + hyperlipidemia + edema. Low C3 and RBC casts point to nephritis.

N16. a, b, c, e — Hypoalbuminemia < 2.5 g/dL; Cholesterol > 200 mg/dL; Generalized pitting edema; Heavy (nephrotic-range) proteinuria
Mnemonic LEAP: Lipids up, Edema, Albumin down, Proteinuria. Gross hematuria is not typical.

N17. b — Urine protein becoming negative / trace for 3 days
Remission = proteinuria disappears; weight falls as edema resolves.

N18. b — Spontaneous bacterial peritonitis
Infection (pneumococcal peritonitis) is a major complication because of IgG loss and steroid therapy.

N19. a, b, c, e — Daily weight at the same time; Measure abdominal girth; Turn frequently and protect edematous skin; Keep away from children with infections
Salt is restricted; protein intake is normal.

N20. b — Thrombosis (DVT, renal vein)
Mnemonic 'I THE H': Infection, Thrombosis, Hypovolemia, Electrolyte, Hyperlipidemia.

N21. a, b, c, d — Onset below 1 year or above 10 years; Persistent hypertension; Low C3; Gross hematuria
Typical MCNS (2–6 yrs, selective proteinuria, normal C3) is treated without biopsy.

N22. b — Test urine for protein daily and avoid live vaccines
Home dipstick detects relapse early; live vaccines are avoided during high-dose steroids; never stop steroids abruptly.

N23. a — In the first 3 months of life
Congenital: first 3 months; infantile: 3–12 months; childhood: after 1 year.

N24. b — 4–6 weeks of daily prednisone
Resistant cases need biopsy and other immunosuppressives.

N25. a — ≥ 2 relapses within 6 months or ≥ 4 in 12 months
Steroid-dependent = relapse during taper or within 2 weeks of stopping.

N26. b — Decreased plasma oncotic pressure from hypoalbuminemia
Albumin loss → fluid shifts to interstitium; secondary Na retention.

N27. b — Periorbital in the morning
Periorbital in the morning, shifts to dependent parts during the day; may become generalized (anasarca).

N28. b — The liver increases lipoprotein synthesis in response to low albumin
Compensatory hepatic synthesis plus reduced lipoprotein lipase.

N29. a, b, c, d — Hypertension; Hyperglycemia; Cushingoid face and weight gain; Growth retardation
Also infection, cataract, osteoporosis, mood changes.

N30. b — Monitor for fluid overload: BP, respiration, crackles
Albumin draws fluid into vessels; risk of pulmonary edema/hypertension.

N31. a — Pneumococcal vaccine
Pneumococcus is the main cause of peritonitis in NS.

N32. a — Excess fluid volume
Physiological needs first; also risk for skin breakdown and infection.

N33. a — Sore throat 1–2 weeks ago
Throat infection precedes APSGN by 1–2 weeks; skin infection (impetigo) by 3–6 weeks.

N34. a, b, c, e — Cola / tea-colored urine; Periorbital edema; Hypertension; Oliguria
HEAL: Hematuria, Edema, Azotemia/oliguria, Lab low C3 + hypertension. Proteinuria is mild–moderate.

N35. b — Notify the physician — suspect hypertensive encephalopathy
Hypertensive encephalopathy (headache, vision changes, convulsions) is an emergency complication.

N36. a — Low C3 with raised ASO titre
C3 is low and returns to normal within ~8 weeks; ASO/anti-DNase B show recent strep.

N37. b — Previous day's urine output + insensible loss (≈400 mL/m²/day)
Intake = output + insensible loss, with salt restriction (DRIP: Diet & Rest).

N38. b — Increased urine output (diuresis)
Diuresis comes first; microscopic hematuria may persist for months.

N39. a, b, c, d — Daily weight and strict I&O; BP monitoring every 4 hours; Low-salt diet; Bed rest during the acute oliguric phase
Potassium is restricted when oliguric (risk of hyperkalemia).

N40. a — C3 still low after 8 weeks or normal at onset
Biopsy if: rapid progression, normal C3, hematuria > 6 months, persistent low C3.

N41. b — Immune complex deposition in the glomeruli
Antigen–antibody complexes deposit → complement activation → inflammation (type III hypersensitivity).

N42. b — Salt and water retention from decreased GFR
Reduced GFR → fluid overload → hypertension, edema, circulatory congestion.

N43. a, b, c — Hypertensive encephalopathy; Acute heart failure; Acute kidney injury with hyperkalemia
DRIP: ICU if renal failure, heart failure or encephalopathy.

N44. b — Eradicate remaining streptococci and prevent spread
Penicillin (or erythromycin) does not change the renal course.

N45. a — Most children (> 95%) recover completely
Gross hematuria clears in 1–3 weeks; microscopic hematuria can last up to 1–2 years.

N46. b — Bananas and oranges
High-potassium foods are restricted during oliguria.

N47. a, b, c — Gross hematuria; Hypertension; Low C3
Massive proteinuria and hyperlipidemia belong to nephrotic syndrome.

N48. a — Daily weight on the same scale
Combined with strict intake and output.

N49. b — Escherichia coli
E. coli causes ~80% of UTIs ('KEEPS' bugs).

N50. b — Fever or temperature instability, poor feeding, jaundice
Infants show non-specific signs: fever, failure to thrive, vomiting, jaundice.

N51. a, b, c, d — Vesicoureteral reflux; Posterior urethral valves; Uncircumcised male infant; Constipation
Obstruction, VUR, hurried micturition, uncircumcised males, constipation.

N52. b — Any bacterial growth
Any growth on SPA/catheter = UTI; clean-catch needs > 100,000 CFU; > 1 organism = contamination.

N53. b — Void regularly and completely; drink enough fluids
PREVENT UTI: hydration, regular complete voiding, treat constipation, wipe front to back.

N54. a, b, d — High fever with rigors; Loin pain / tenderness; Vomiting and toxic look
Cystitis: dysuria, urgency, frequency, little or no fever.

N55. c — Urine bag
Bag specimens are useful mainly when negative.

N56. a — Recurrent pyelonephritis and renal scarring
Scarring can lead to hypertension and chronic kidney disease.

N57. a, b, c, e — Age below 3 months; Toxic appearance or vomiting; Suspected pyelonephritis with dehydration; Urosepsis
Older children with cystitis can be treated orally.

N58. b — Clean the genital area, discard the first part, then collect midstream
Send to the lab within 1 hour or refrigerate.

N59. a — Positive leukocyte esterase and nitrites
Pyuria (> 5 WBC/HPF) and bacteriuria; culture confirms.

N60. a — Low-dose nitrofurantoin or TMP-SMX at night
Given once at bedtime; plus treating constipation and regular voiding.

N61. a — Pre-renal
Pre-renal = decreased perfusion (dehydration, shock, hemorrhage).

N62. b — Calcium gluconate 10% slowly IV
Calcium gluconate stabilizes the myocardium first; then shift K (insulin+glucose, bicarbonate, salbutamol) and remove it.

N63. a, b, c, d — Fluid overload with pulmonary edema; Refractory hyperkalemia; Uremic encephalopathy; Severe metabolic acidosis not responding
A-B-C-U-R plus refractory acidosis and hyperkalemia.

N64. b — Strict intake and output with daily weight
Fluid balance guides therapy; restrict K and fluids.

N65. a, b, c, e — Raised urea and creatinine; Metabolic acidosis; Hyperkalemia; Hypocalcemia
Phosphate rises (hyperphosphatemia) with low calcium.

N66. c — Post-renal (obstructive)
Obstruction distal to the kidney = post-renal; ultrasound first.

N67. a — 0.5–1 mL/kg/hr
< 1 mL/kg/hr in infants, < 0.5 mL/kg/hr in children.

N68. a, b, c, e — Aminoglycosides (gentamicin); NSAIDs; Contrast media; Amphotericin B
Monitor drug levels and creatinine; ensure hydration.

N69. a — Hemolytic anemia, thrombocytopenia and renal failure after bloody diarrhea
Usually E. coli O157:H7 — an intrinsic renal cause.

N70. a — Peaked (tall tented) T waves
Later: widened QRS, sine wave, arrest.

N71. b — Dehydration and hypokalemia
Large urine volumes cause water and electrolyte loss.

N72. a — It is technically simple and needs no vascular access
Watch for peritonitis: cloudy effluent, fever, abdominal pain.

N73. a, b, c, e — Adequate calories mainly from carbohydrate and fat; Restricted potassium; Restricted phosphate; Controlled protein
Enough calories prevent catabolism, which raises urea and K.